

**The 4th (2017)
Generation
Surveillance of
Commercial Sex
Workers Female
Patrons and Workers
of Sites where Persons
Meet Sex Partners or
Participate in Sexual
Activity in Jamaica**

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Acronyms

AIDS	Acquired Immune Deficiency Syndrome
BBS	Bio-Behavioural Surveillance
CSW	Commercial Sex Worker
FSW	Female Sex Worker
FP	Female Patron
HIV	Human Immunodeficiency Virus
MoH	Ministry of Health
MSM	Men who have Sex with Men
NHP	National HIV Programme
PLACE	Priorities for Local AIDS Control Efforts
PLHIV	People Living with HIV
PW	PLACE Worker
S&D	Stigma and Discrimination
SRH	Sexual and Reproductive Health
UNAIDS	United Nations Program on HIV/AIDS
USAID	United States Agency for International Development
UWI	University of the West Indies

TITLE: The 4th (2017) Generation Surveillance of Commercial Sex Workers Female Patrons and Workers of Sites where Persons Meet Sex Partners or Participate in Sexual Activity in Jamaica

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Executive Summary

The report documents the findings of the 4th generation bio-behavioral (BBS) surveillance survey (2017) of female sex workers (FSW) female patrons (FP) and PLACE workers (PW) at sites where persons meet sex partners or participate in sexual activity in exchange for money in Jamaica. The research protocol reflects only minor changes to the 2011 and 2014 PLACE survey protocols to establish HIV and STI prevalence among FSW, PW and FP for the purpose of guiding and improving HIV/STIs interventions for these women and to achieve the following objectives:

- I. To determine prevalence and incidence of HIV and Syphilis among high risk women.
- II. To measure proximate & underlying determinants of HIV and STI transmission
- III. To monitor program coverage
- IV. To establish closer contacts with key populations in order to expand prevention and treatment services
- V. To determine effectiveness of prevention interventions among FSW
- VI. To estimate female sex workers population size
- VII. To understand social and sexual networks of FSW

The study was carried out between August and November 2017 in the four health regions, South East, North East, Southern and Western. Data collection consisted of three phases:

- Phase One - Community informants listed places where people meet new sex partners. A total of 585 sites across 11 parishes in Jamaica were identified as places where people meet new sex partners.
- Phase Two -Visits were made to the sites to verify or reject these locations and specifically identify the sites where sex in exchange for money took place. Among these sites 224 were verified as places where female sex workers solicited clients. The numbers of females recorded during the site verification at the busiest times were 1,258 FSW, 980 exotic dancers and 858 PWs.
- Phase Three - A sample of 1238 FSW, PW and FP aged 16 and older were enrolled in the study where knowledge attitudes and behaviour data was collected along with biological

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samples from each respondent that were tested for HIV and Syphilis. Street sex workers had the greatest representation in the sample (24.9%, 307) followed by sex workers at the clubs, massage parlours etc. (21.6%, 267) and dancers (18.7%, 231). Workers including female bartenders, waitresses (14.5%, 179) and female patrons (20.5%, 254) also made up the participant sample. Of the 11 parishes that were surveyed, St. Catherine had the greatest representation of the sample at 25% (309) followed by St Ann at 20.2% (250) and the twin parishes of Kingston and St. Andrew with 14.8% and 2.3% respectively then Trelawny as a single parish with 2.8% (35) respondents. Interviews were not conducted in St. Mary, Portland and St. Thomas.

Demographics

- Participants' ages ranged from 16 to 68 with a median age of 28 and a mode of 22. Reported age of sexual initiation ranged from as low as 8 years old to 35 years of age. The mean age of sexual initiation was 15.6 while the modal and median age was 16. The standard deviation was 2.299.
- Less than 50% of the women had completed high school and only 2% had tertiary education.
- Union status among participants varied widely, 33.5% (409) stated they were currently in a union, 19.8% (242) were previously in a union and 46.6% had never been in a specified union. Approximately 70% (855) of participants reported currently having a main male partner while 14% (167) said they had a main female sex partner.

Cash for Sex

- All categories of participants said they received cash for sex. Overall 62.6% said they exchanged cash for sex in the last 6 months while 65.6% said they received cash for sex in the last 3 months. As expected sex workers were more likely to admit to paid sex with an average of 20% of patrons and workers admitting to paid sex in the last 6 & 3 months.
- An average of one third of sex workers and exotic dancers knew a family member who also exchanged cash for sex.

Condom Use

- There was no difference between FSWs, patron or workers re condom use with main sex partner at last sexual encounter as 32% of street sex workers, 27% of other sex workers, 23% of dancers, 34% of workers and 27% of patrons reported to have done so $p=0.890$

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Overall 29.3% of participants said they used a condom at last sexual encounter with their main partner.

- Last sexual encounter condom use with paying clients was much higher among sex workers than patrons and workers. There was reported 96.7% use among other' SW followed by street SW (96%), dancers (91.2%), workers (62.5%) and among patrons (40.5%). Overall 66.6% (805) of participants reported using condoms at their last paid sexual encounter.

HIV and Syphilis Rates

- Biodata revealed new STI prevalence rates among Jamaican Sex workers, 2% (16) of the 805 sex workers were HIV positive and 3% (30) were reactive for syphilis. None of the 254 patrons in the survey tested positive for HIV and 3 or 1% were reactive for syphilis. Meanwhile 1.7% (3) of the 179 workers tested HIV positive while none were reactive for syphilis.

Drug and Alcohol Use

- Alcohol was the most reported substance used by respondents in the past 4 weeks with an average of 83.32% of participants having used. Street sex workers recorded the highest use of crack cocaine among the categories of participants (8%) followed by other sex workers 5.3% and dancers 3.1%. Workers and patrons were the least likely to use crack cocaine Half or more than half of sex workers and exotic dancer used marijuana while 37% of workers and 39.3% of patrons also used that substance. The use of ecstasy was highest among exotic dancers at 25.6% followed by 20% of street sex workers and 13% of other sex workers.

Adverse Events

- Sex workers reported experiencing a number of adverse events. Spending a night in jail was most commonly reported among sex workers with 40.6% of street sex workers, 34.5% of dancers, 24.1% of other sex workers, 18.8% of workers and 17.1% of patrons having spent a night in jail or in prison. Almost one-quarter (22.7%) of SW reported being raped, with street SW and dancers recording the highest incidence of rape. Reported violent attacks by clients were highest among street SW (38.5%) followed by other SW (27.2%), dancers (22.9%), workers (10.5%) and patrons (5.6%). Having slept

outside or in a shelter was the least reported adverse event with 23.4% of street sex workers, 18.0% of other SW, 17.0% of dancers, 11.4% of workers, 8.0% of patrons experiencing this event.

Exposure to HIV Prevention Activities and Materials

- Sex workers have higher levels of exposure to condom demonstrations, risk reduction conversations and HIV prevention materials than other categories of participants. For example while 68.9% of street SW, 71.8% of other SW and 70.2% of dancers had participated in condom demonstration only 56.2% of workers and 53% of patrons had.
- Overall 79% of participants had been tested in the last twelve months excluding the test conducted during the survey. Exotic dancers were the most frequently tested in the last 12 months with 87.8% having done so while 82.2% of street sex workers, 77.8% of other sex workers, 74% of workers and 70.4% of patrons had also done so in the last 12 months

Programme Implications

- HIV prevalence among female sex workers seems to be continuing to steady decline from 4.9% in 2008, 4.1% in 2011, 2.8% in 2014 and 2% in this 2017 survey.
- Syphilis also appears to be on the decline with 3 % of sex workers and 1% of patrons being reactive compared to 6.3% in 2014.
- In the 2014 PLACE surveillance 2.8%% of sex workers were HIV positive 90.9% had neither HIV nor Syphilis while 0.7% were infected with both HIV and Syphilis. In 2014, 6.3% were infected with syphilis but were negative for HIV while 2.1% were HIV positive but not reactive for syphilis. In 2017 2 %% of sex workers were HIV positive and 94.3% of all sex workers had neither syphilis nor HIV while 0.7% or 6 were infected with both HIV and Syphilis. In 2017, 2.9% (24) were infected with syphilis but were negative for HIV.
- Partner/ relationship intimacy continues to be the biggest threat to consistent condom use.
- The difference between interviewer elicited response to last time condom use with paying partners (76%) and participants privately listed response (58%) is of note and should be borne in mind during risk reduction conversations.
- Drug and alcohol use continues to be high and is used to cope with the physical and psychological stresses of sex work.

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The steady decline in the prevalence of HIV among sex workers is rewarding and reflects the substantial investments made to reach sex workers and the effectiveness of the interventions targeting FSWs. However, it is possible that the sex workers reached and tested in this survey might be exclusive of other categories of sex workers who are not captured in the radar both of this study and general targeted interventions and are less readily identifiable. It is important that regular outreach for sex workers seeks to identify and target these women as well as those in the traditional sites included in this survey.

The proximate determinant posing the biggest threat to consistent condom use in this survey was the inconsistent use of condoms with main partners and regular paying clients due to partner/relationship intimacy. Incomplete education and early child bearing were also found to be socio-economic drivers of entry into sex work. Vulnerability created by poverty and lack of social support was also evident by the adverse events recorded among participants.

Programme coverage was reflected in the high levels of exposure to HIV prevention messages among FSWs and access to HIV testing was the most evenly provided aspect of the HIV programme as almost 88% of exotic dancers received an HIV test in the last 12 months and 70% of patrons. Since workers and patrons at sites where sex workers operate also exchange sex for money they need to receive similar prevention interventions as the sex workers as the endorsement of some myths such as a healthy looking person cannot have HIV and an unwillingness to share a meal with someone who is HIV positive still remain too high. Appropriate treatment seeking behaviours may also indicate programme effectiveness but the use of home remedies to treat STIS was still a feature among this population.

Recommendations

- It is vital the successes achieved among the female sex workers that have been reached be maintained through adequate allocation of resources for prevention interventions with this population. It is clear that the patrons and workers who are also associated with the sites where sex workers and exotic dancers solicit are also involved in transactional sex. Similar levels of intervention should be provided to these women.

- Ways must be found to adequately address the threat partner intimacy to condom use. It is possible to reach the regular client and main partners of sex workers in order to have them assess their own risk of HIV/STI infection. This is necessary as the women are more likely to use condoms with these partners if the men are open to or request condom use.
- The use of alcohol and drugs must be addressed consistently particularly the use of ecstasy among exotic dancers and crack cocaine among street sex workers. The slight trend towards the use of IV drugs needs to be monitored closely and appropriate interventions pro-actively provided.
- The underlying determinants and vulnerability of female sex workers need to be addressed to prevent risk taking in times of desperation and make the reduction in number of partners be possible particularly among street based sex workers. Their heavy reliance on sex work for the main source of income and more adverse life events put them at greater risk. Ways must be found to provide more social support for those most at risk by creating linkages to government and non- governmental social agencies.
- Increase STI education in order to promote appropriate treatment seeking behaviours among all sex workers as well as the workers and patrons.

Background

Sex workers (SWs) and men who have sex with men are the key populations at the highest risk of HIV infection due to their specific social and behavioural characteristics (1). In the countries like Jamaica with both low-level and concentrated HIV epidemics, these populations account for most HIV infections and are also called “core groups” for HIV transmission (2). Various data sources have shown that there are a number of factors contributing to the heightened risk among FSW, PW and FP, which if left unattended, have the potential to erode the gains made in HIV/STI prevention and control efforts at the national level (1, 2, & 3). These include lack of consistent condom use among sex workers, especially with their regular clients and main partners. Furthermore, some of these behaviours are driven by increasing poverty, population dynamics, and inadequate academic certification among SW. The PLACE (2014) surveillance report (4) established that only 27.8% of the street sex workers had completed grade 11 and 48.8% of the other sex workers. Most of the women had children (64%-78.1%) and Most of the women were in the 25-39 age groups with a large number of street and club based sex workers having the experience of having been jailed. Additionally, many reported that they had no employment (17.4%) or were only employed part-time (31.5%). These realities are likely to have exacerbated the negative impact of general socio-economic challenges on the population of sex workers and further increased risky sexual practices. The current 2017 bio-behavioral surveillance data is critical in informing policies and programs to better respond to the socio-economic determinants and sexual health risks of sex workers.

There are a number of success stories to report on in the National response to HIV. Jamaica was recently among 20 countries in the world and specifically three in Caribbean to have reduced HIV incidence by more than 50% between 2001 and 2012 with fewer persons currently contracting HIV (new HIV cases declined by 42% as of 2013 according to UNAIDS 2014 estimates) than in previous years (5). The country has the highest proportion of people living with HIV who know their status in the English-speaking Caribbean—an achievement attributed to community-led prevention and testing programmes and wide spread of provider initiated testing (6). Access to Antiretroviral medication (ARVs) and have probably never been more robust and strategic and more persons living with HIV (PLHIV) are being provided with better access to treatment (7). However there remains an urgent need to continually improve current

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prevention programmes to limit the spread of HIV as a myriad of approaches are required to contain the epidemic. They include school-based programs, mass media campaigns and targeted approaches for those most at risk. Prevention efforts also need to be driven by current information about the HIV and STIs epidemics in high risk groups in order to also reduce sexual transmission of HIV in the general population. The Caribbean is the second most affected region with respect to the HIV/AIDS epidemic with an adult HIV prevalence of 1.1% as of 2013 (4). Jamaica's prevalence of 1.7% in comparison exceeds the regional estimate and is disproportionately affected by HIV. As of 2016, 30,904 Jamaican adults were diagnosed with HIV infection, of which 9821 (27.4%) persons are known to be deceased. Patterns of risk have remained the same over the past few years with multiple sex partners and a history of STIs most reported among people diagnosed with HIV infection (8). Sentinel surveillance of women attending antenatal clinics indicate that there is no significant change in the prevalence of HIV over the last decade despite prevention efforts, which stands at 1% (2013). Research has shown the odds of FSW contracting HIV compared to other women was 13.5:1 (9). The global HIV prevalence among sex workers was estimated to be about 12% (10). Despite the HIV rate among sex workers in Jamaica being lower than the global prevalence and a marginal decline in HIV prevalence among sex workers from 4.9% in 2008 to 4.1% in 2011, and 2.8% in 2014 about 1 in every 10 new HIV cases in 2012 was estimated to have been associated with commercial sex (6). The population of sex workers increased from approximately 14,957 in 2011 (UNAIDS, 2013) to 18,696 sex workers in 2014 (4).

The current 2017 bio-behavioural surveillance survey among FSW, PW and FP will be the sixth since 1990 and is one of the main data sources used in the design and implementation of HIV prevention and control programs, targeting sex workers as well as monitoring and evaluating the effectiveness of these programmes.

The purpose of this report is to document the findings of the 4th generation bio-behavioral (BBS) surveillance survey (2017) of female sex workers (FSW) female patrons (FP) and PLACE workers (PW) at sites where persons meet sex partners or participate in sexual activity in exchange for money in Jamaica. The findings will assist in guiding and improving strategies and HIV/STI interventions tailored to the needs of FSWs, PWs and FPs at risk for HIV.

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Methodology

The 4th Generation Cross-sectional bio-behavioral surveillance (BBSS) among FSW, other PW and FP at sites where people meet new sexual partners was approved by the MOH Ethics Committee. The research protocol reflected only minor changes to the 2011 and 2014 PLACE surveys to establish HIV and STI prevalence among FSW, PW and FP at risk and to gather information to continue to guide and improve HIV/STIs interventions among these women. (See Appendix 1 for Full Research Protocol)

The specific objectives of the study were:

- I. To determine prevalence and incidence of HIV and *Syphilis* among high risk women.
- II. To measure proximate & underlying determinants of HIV and STI transmission
- III. To monitor program coverage
- IV. To establish closer contacts with key populations in order to expand prevention and treatment services
- V. To determine effectiveness of prevention interventions among FSW
- VI. To estimate female sex workers population size
- VII. To understand social and sexual networks of FSW

The study was carried out between August and November 2017 in the four health regions, South East, North East, Southern and Western. No data was collected in phase one and two for St. Mary and Portland. Data collection consisted of three phases in keeping with the PLACE methodology. In Phase one community informants listed places where people meet new sex partners while in phase two visits were made to verify or reject these locations and specifically identify those locations where sex was sold and bought. In phase three a sample of 1238 females aged 16 and older were enrolled in the study that included female sex workers (FSW) other PLACE workers (PW) and female patrons (FP). Knowledge attitudes and behaviour data was collected. HIV and Syphilis tests were conducted on each participant.

. The study aimed to obtain evidence to address the following BBS research questions:

1. What are the characteristics of venues where people meet new sexual partners in Jamaica?

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2. Where are venues located?
3. What evidence is there of HIV prevention program efforts at the venues?
4. Are condoms available onsite?
5. Are HIV prevention messages displayed onsite?
6. What is the prevalence and incidence of HIV and STIs among women at venues identified as places where people meet new sexual partners?
7. What is the prevalence and incidence of HIV and STIs among women who exchange sex for money?
8. What are the socio-demographic and behavioral characteristics of these groups?
9. What is the extent to which these groups have access to HIV prevention programs?
10. What are the characteristics of those infected with HIV?
11. What are the characteristics of those infected with other STIs?

Results

PLACE Community Informant and Site Verification Data Report

A wide variety of sites emerged from the Community Informant and Site verification phases as places where people meet new sex partners. A total of 585 sites were verified for 12 of the 14 parishes in Jamaica as places where people meet new sex partners; of these 224 were confirmed as places where female sex workers solicited. The number of women recorded during the site verification at the busiest times were 1,258 FSW, 980, female exotic dancers and 858 PWs.

The parish with the largest number of sites was St Catherine with 89 sites (table 1) of which almost half were not sites frequented by sex workers. St Ann had the second largest number of sites where people meet new sex partners (75) of which 40 were not sites frequented by sex workers. Hanover and St Andrew which had the third and fourth largest number of sites respectively followed a similar pattern. The sites selected for the survey were those with the largest number of sex workers at the busiest times. In total 119 sites were included in the survey.

Table 1. Survey Sites

Parish	Number of sites Verified	Number of Sex work sites	Number of Survey Sites
Kingston	37	5	3
St Andrew	59	14	13
St Catherine	89	48	29
St. Ann	75	35	23
Westmoreland	34	19	14
Hanover	62	17	14
St. James	42	27	16
Trelawney	35	12	7
St. Elizabeth	51	17	8
Clarendon	30	12	7
Manchester	55	22	6
St Thomas	16	2	0
Total	585	224	119

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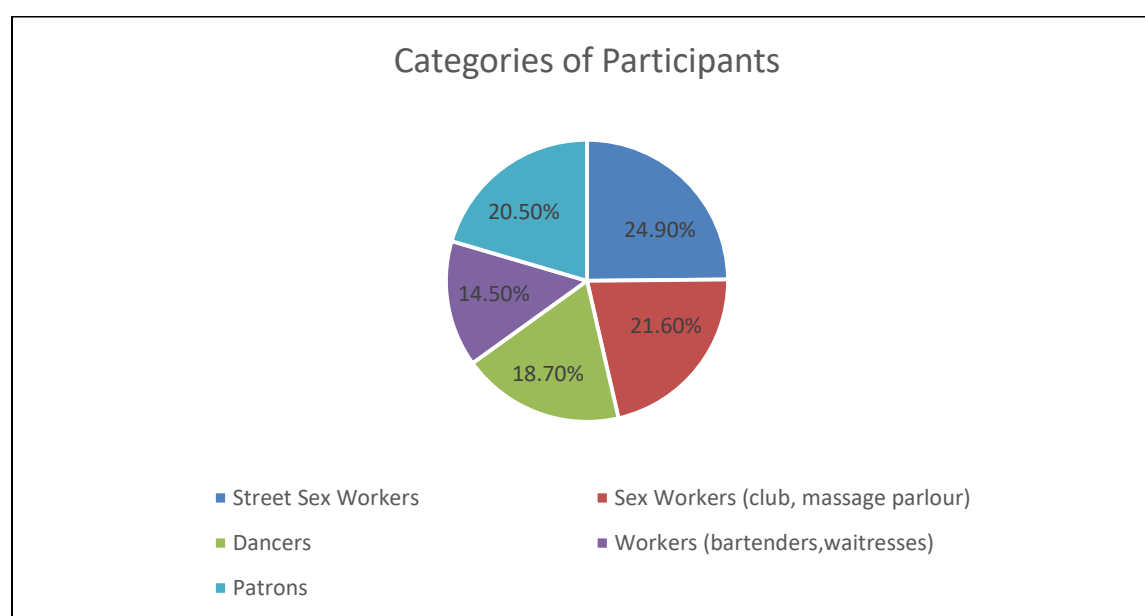
Most sites that were verified as frequented by sex workers had condoms available ranging from 90% in St Ann to 52% in St Andrew.

HIV Prevention messages were visible at less than one third of the sites. For example of the 48 sites verified as frequented by sex workers in St Catherine 11 had HIV prevention messages displayed while in St Ann 17 of the 35 sites frequented by sex workers had such messages displayed.

PLACE Survey Data Report

A total 1238 participants were interviewed as part of the survey. Street sex workers had the greatest representation in the sample (24.9%, 307) followed by sex workers at the clubs, massage parlours etc. (21.6%, 267) were the population. Dancers (18.7%, 231), Workers including bartenders, waitresses (14.5%, 179) and Patrons (20.5%, 254) also made up the participant sample.

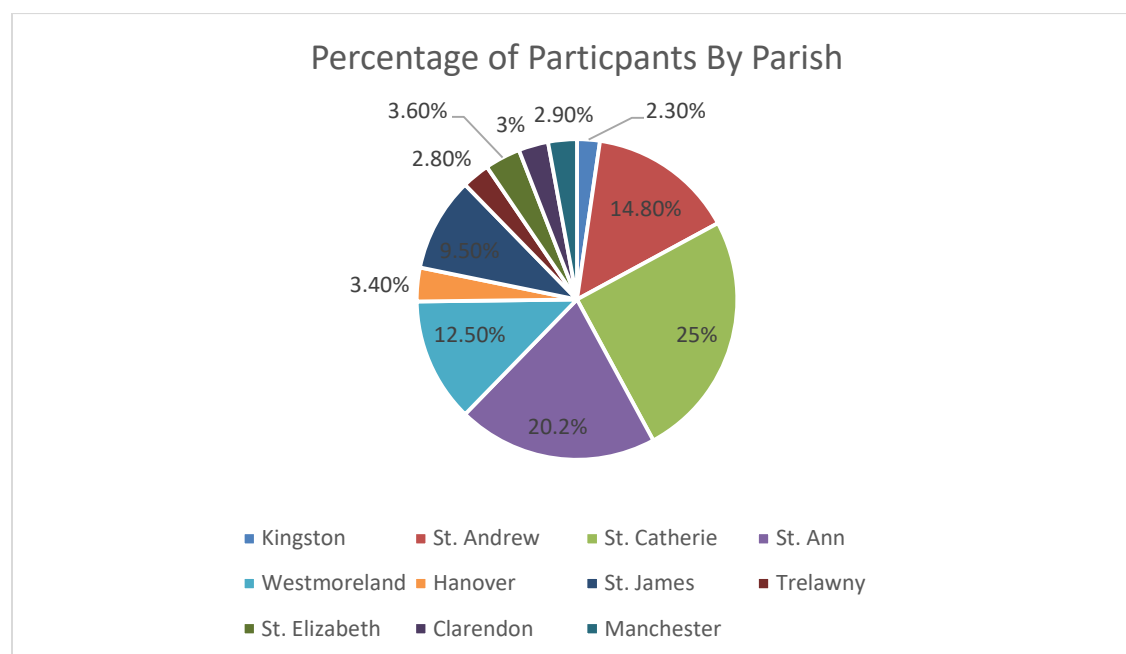
Figure 1. Categories of Participants



Parish of Interview

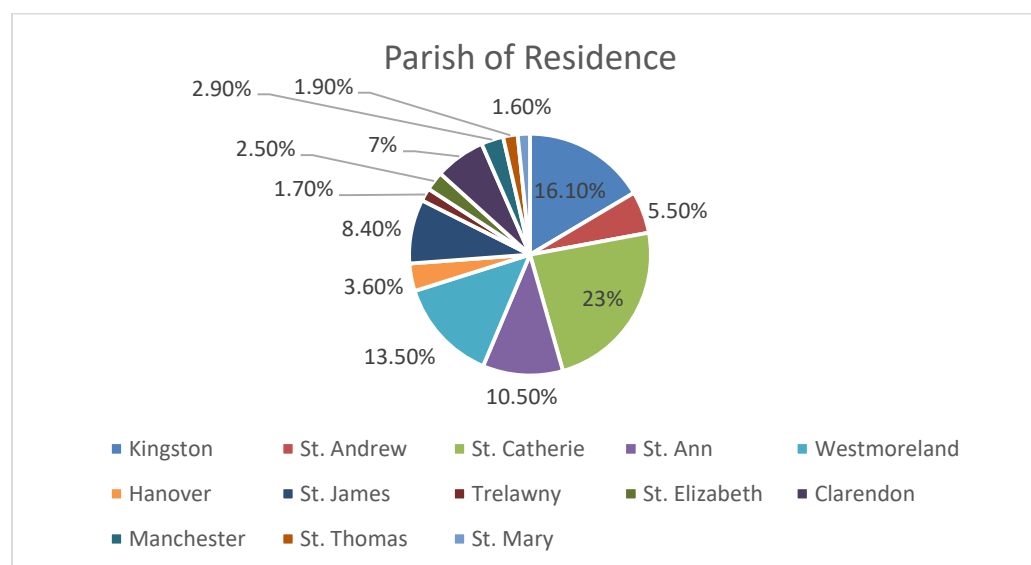
The survey was conducted in 11 of Jamaica's 14 parishes. St. Catherine had the greatest representation of the sample at 25% (309) followed by St Ann at 20.2% (250) while Trelawny as a single parish had 2.8% (35) respondents. The twin parishes of Kingston and St. Andrew had 14.8% and 2.3% representation respectively. Interviews were not conducted in St. Mary, Portland and St. Thomas.

Figure 2. Participants by Parish



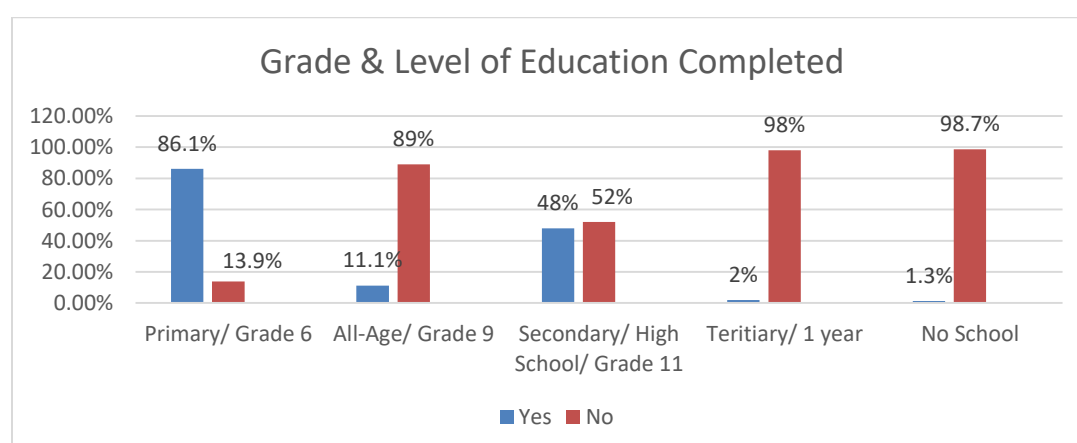
Gender, Age, Union Status and Parish of Residence

Respondents' ages ranged from 16 to 68. The median age was 28 and the mode 22. A total of 1,220 participants stated their union or marital status with 33.5% (409) stating they were currently in a union, 19.8% (242) previously in a union and 46.6% had never been in a specified union. The largest proportion of participants 23% resided in St. Catherine followed by Kingston 16% then Westmoreland and St Ann with 13.5% and 10.5% respectively (figure 3).

Figure 3. Parish of Residence

Education

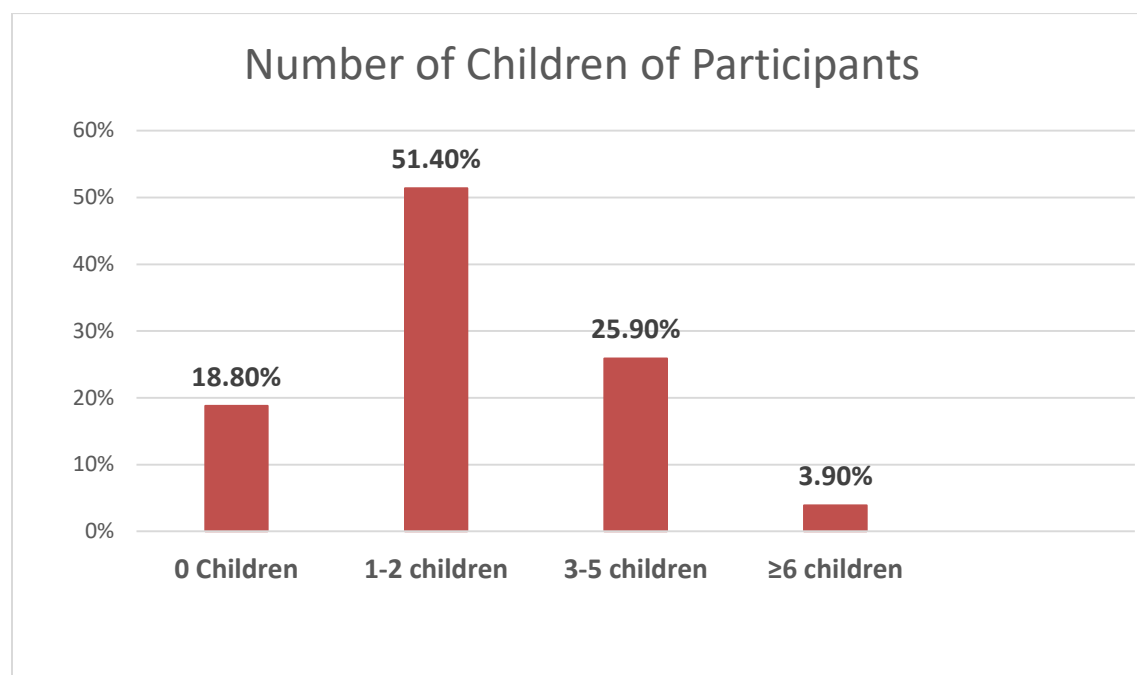
Respondents were asked to state the highest level of education completed in primary school, in secondary school, and at post-secondary tertiary level. The highest number of respondents gave Primary school 86.1% (1030 of 1196) with grade 6 being the highest grade complete. Less than 50% had completed High school 48.4% (565 of 1167); 11.1% (113 of 1019) completed All-age school at grade nine; 3.8% (47 of 1225) reported having tertiary education while 3.9% had vocational/ skills training A total of 16 or 1.3% said they attended no school at all (figure 4).

Figure 4. Level of Educational Attainment

Number of Children

The mean number of children reported by participants was 2 while the median and mode were 1 child as the number of children ranged from 0 -12 with 18.8% (218) having no children. The proportion of participants with 6 or more children was 3.9% (44) as only 1,158 of the 1,238 participants responded to this question. Most respondents 51.4% (595) reported having between 1-2 children (figure 5).

Figure 5. Number of Children of Participants



Place of Operation

Most Street Sex Workers 76.9% (307) operated at a Street Sex Work site while 60.3% of Other Sex Workers operated at an exotic club with dancers (table 2). Most of the non- street based sex workers operated from exotic clubs 60.3% (161) followed by massage parlours 12% (32).

Table 2. PLACE Sites

		Street Sex Worker	Other Sex Worker (club, massage parlours etc)
Type of site	Bar	0.7% (2)	3.7% (10)
	Outdoor Hangout Spot	9.4% (29)	1.9% (5)
	Sports Bar	1.3% (4)	7.9% (21)
	Night Club	1.0% (3)	7.9% (21)
	Exotic Club with Dancers	9.4% (29)	60.3% (161)
	Massage Parlour	0.0% (0)	12.0% (32)
	Fast Food/ Restaurant	0.0% (0)	0.0% (0)
	Street SW Site	76.9% (236)	1.9% (5)
	Hotel/Motel/Guest House	0.0% (0)	3.0% (8)
	Mall/Plaza	1.3% (4)	1.5% (4)
Total		307	267
		100.0%	100.0%

Length of Time in Sex Work

Approximately 16% (49) of street sex workers reported they had been in sex work for over 10 years with 6.5% (20) stating over 5 years; 19.4% (60) stating 1-5years and 3.2% (10) 6-12 months. Thirty nine or 14.6% of other sex workers reported involvement in sex work for more than 10 years while 8.8% (23) reported being involved for over 5 years; 26.5 (71) 1-5 years and 8.8% (23) 6- 12 months. No dancers had over 10 years of experience. The largest proportion of dancers who admitted to sex work 22.7% (52) reported working in the occupation for 1-5 years followed by 13.6% (31) for 6-12 months.

Age of Sexual Initiation of Participants

Participants reported age of sexual initiation ranged from 8 to 35. The mean age of sexual initiation was 15.6 while the modal and median age was 16. The standard deviation was 2.299.

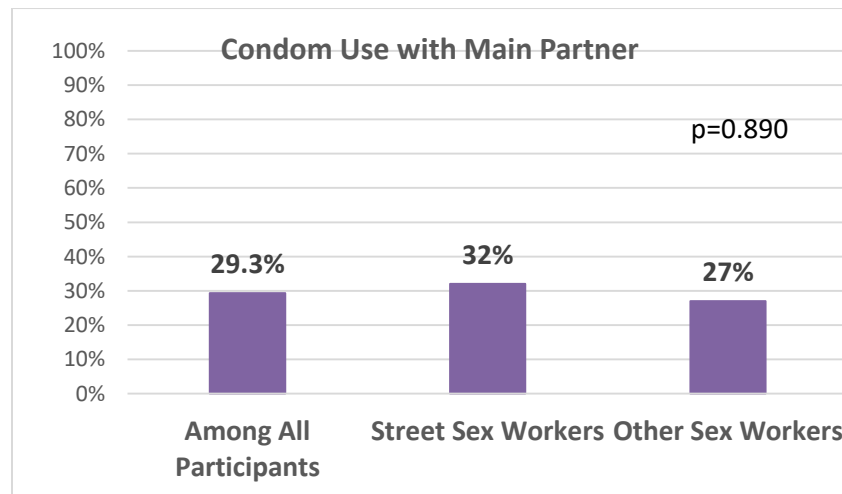
Condom Use with Main Partner

Participants were asked whether they had a main male or female sex partner. Approximately 70% (855) of participants reported having a main male partner while 14% (167) said they had a main female sex partner.

Respondents were asked if they used a condom at last sexual encounter with their main partner and 29.3% affirmed. There was no difference between FSWs, patron or workers re condom use with main sex partner at last sexual encounter as 32% of street sex workers, 27% of other sex workers, 23% of dancers, 34% of workers and 27% of patrons reported to have done so $p=0.890$ (figure 6).

Of the respondents that reported having a main male partner 34.3% said that they used a condom at last sex and 65.4% said that they did not. Of the respondents that reported having a main female partner 33.1% said that they used a condom at last sex and 62.6% said that they did not.

Figure 6. Condom Use with Main Partner



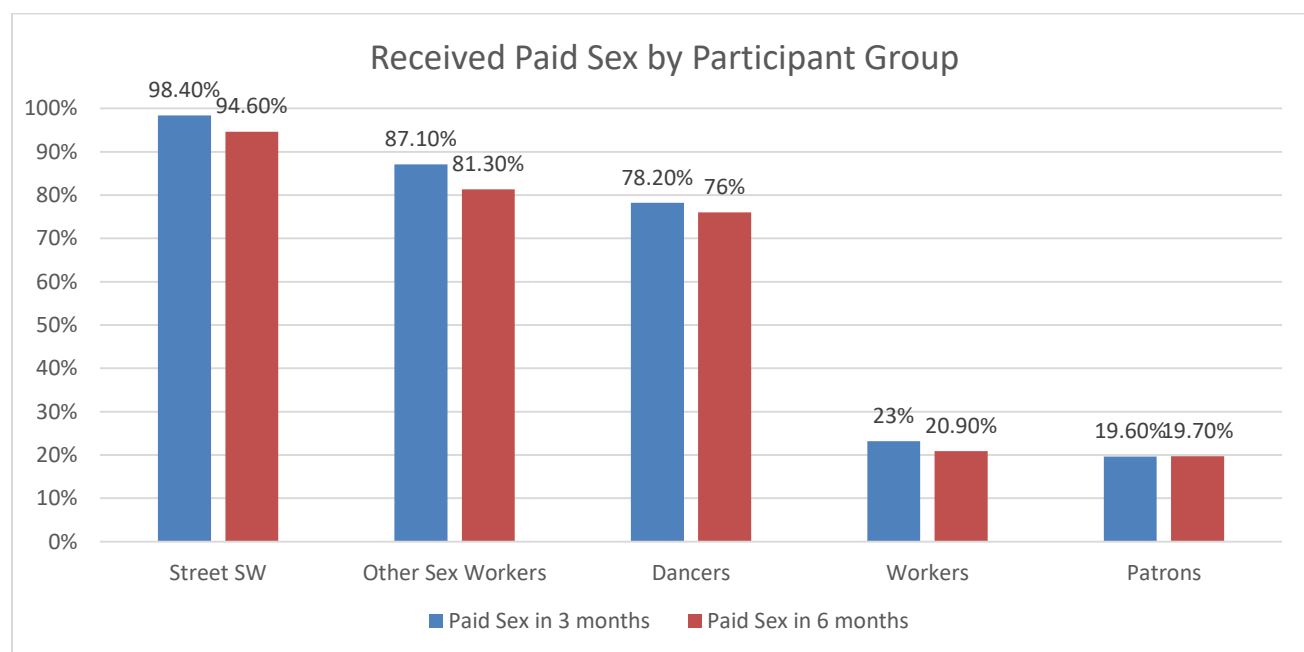
Participants were asked their reason for not using a condom, 58% said they loved or trusted their partner which was the top reason given for no condom use. Don't like using condom was the second top reason given at 6.3%, while 2.1% gave using the pill or other contraceptive as the reason.

Average Number of Sex Partners

The number of partners participants had sexual encounters with was determined by asking respondents to state the number of partners in the last 7 days, 4 weeks and 12 months and the number of new partners in the last 4 weeks. Participants reported number of partners ranged from 0 – 150 (median partners 2) in the last 7 days, 0 – 300 (median partners 6) in the last 4 weeks and 0 – 3000 (median partners 20) in the last 12 months. The number of new sex partners in the last 4 weeks ranged from 0 – 150 (median partners 2). Given the wide range in number of partners, the mean number of sexual partners is unreliable. The median number is more reliable given wide range in numbers.

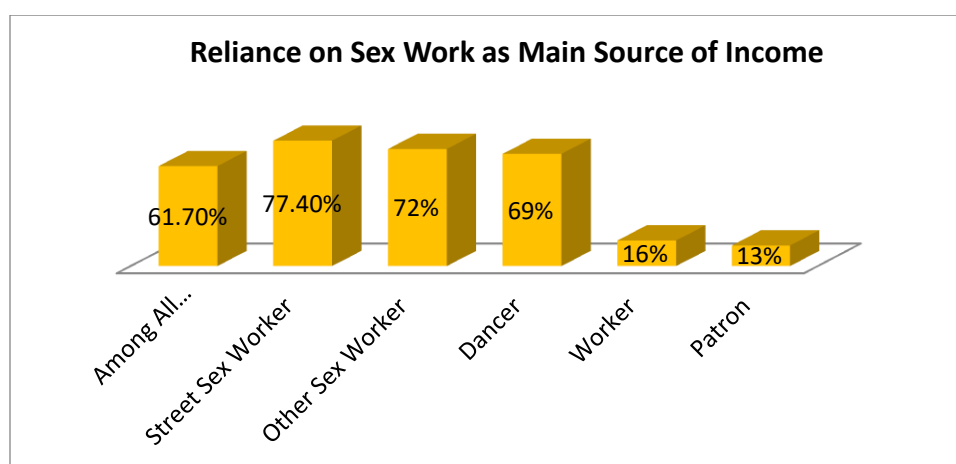
Received Cash for Sex

Participants were asked if they received cash for sex in the last 6 months of which 62.6% responded yes while 65.6% said they received cash for sex in the last 3 months. Among respondent groupings, 98.4% and 94.7% street sex workers said they received cash for sex in the last 3 and 6 months respectively. Similarly, 87.1% and 81.5% other sex workers, 78.2% and 75.2% dancers, 23.2% and 21.0% workers (bartenders etc.), and 19.6% and 19.7% patrons responded in the affirmative receiving cash for sex in 3 and 6 months (figure7).

Figure 7. Received Paid Sex by Participant Group

Sex Work Main Source of Income

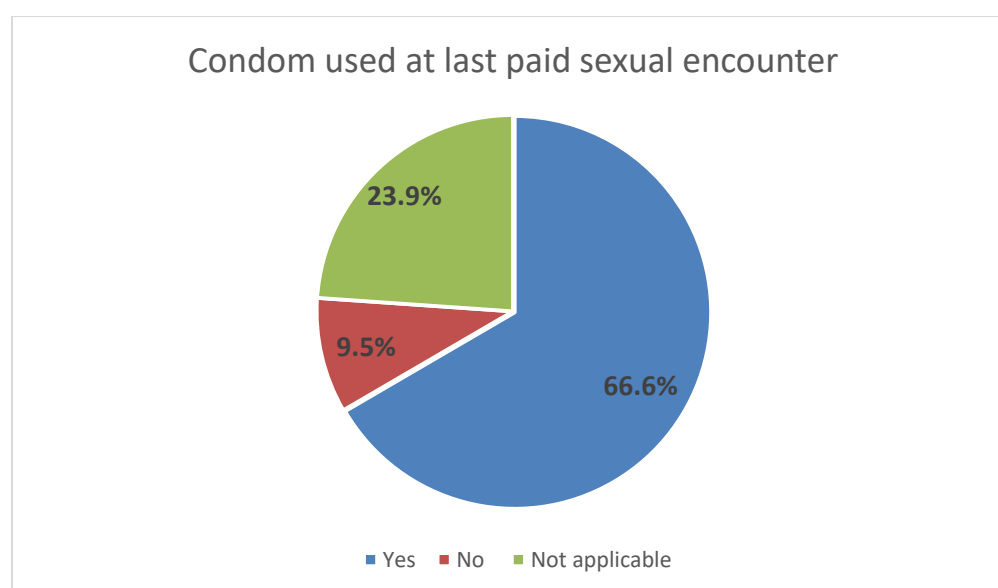
Of the 943 women that responded, 61.3% (578) said that sex work was their main source of income. Among street sex workers, 77.4% (233) reported sex work as their main source of income, 72.3% (180) of other sex workers and 69.3%, (138) of dancers reported the same. Among workers and patrons a lower level of reliance on sex work as a main source of income was reported with 15.8% and 12.7% respectively (figure 8).

Figure 8. Reliance on SW as Main Source of Income

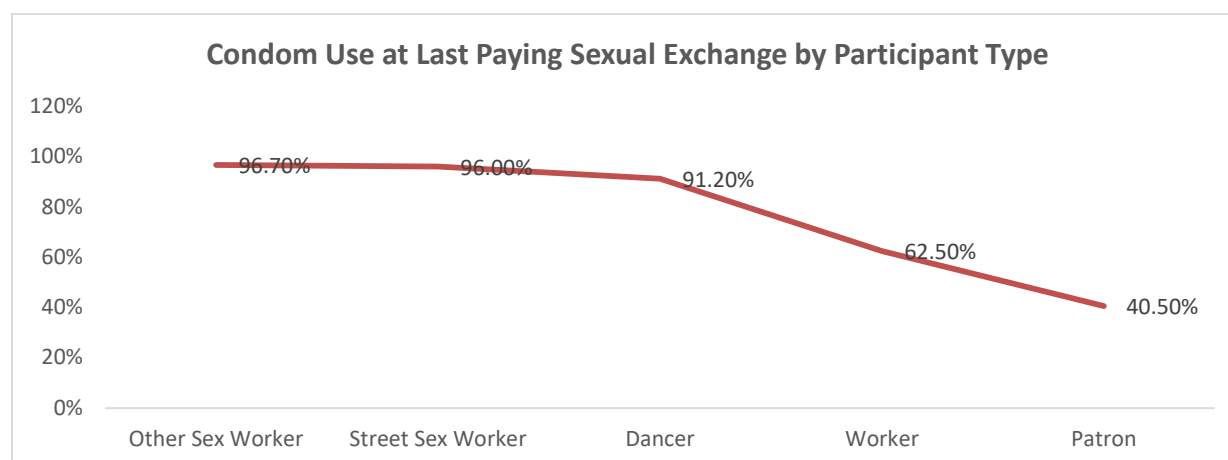
Condom Use at Last Paying Sexual Exchange

To increase the likelihood of open and honest responses, all survey participants were given paper to anonymously and privately write responses to questions which were read to them. One such question asked was *“The last time you paid or were paid money for sex, did you use a condom?”* A total of 1207 participants answered this question 66.6% (805) participants answered yes and 9.5% (115) answered no while 23.9% said the question was not applicable (figure 9).

Figure 9. Condom Use a Last Paid Sex

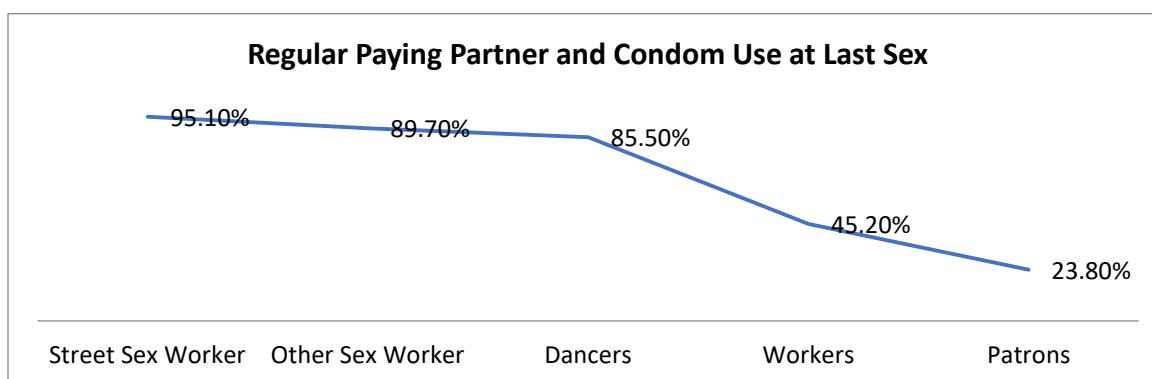


Condom use with in the most recent paid sexual encounter was highest among ‘other’ SW (96.7%), followed by street SW (96%), dancers (91.2%), workers (62.5%) and the lowest condom use reported was among patron (40.5%) in their last paid sexual exchange (figure10).

Figure 10. Condom Use at Last Paying Sexual Exchange by Participant Type

Regular Paying partner and Condom Use at Last Paying Sexual Exchange

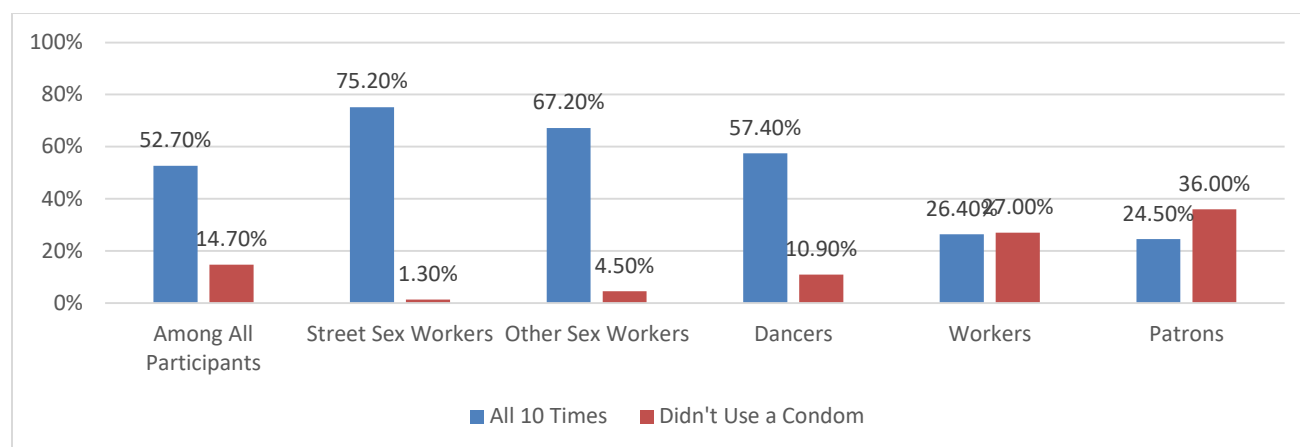
Approximately 76.7% of participants reported using a condom at last paying sexual exchange with a regular client, 8.4% said they had not and 14.9% said the question was not applicable to them (figure 11). Reported condom use with last regular paying client was highest among street SWs (95.1%) followed by ‘other’ SW (89.7%) dancers (85.5%) workers (45.2%) and lowest among patrons (23.8%). (See figure 11).

Figure 11. Condom Use with Regular Paying Partner

Frequency of Condom Use

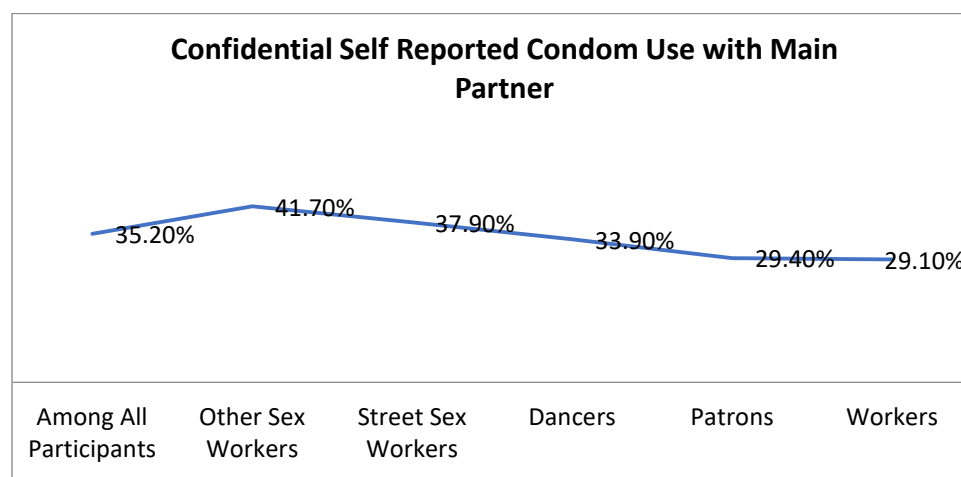
Participants were asked to reflect on their frequency of condom use in their last 10 sexual encounters. Responses ranged from never at all, 1-2 times, 3-5 times, 6-7 times, 8-9 times and all 10 times. Among all participants 52.7% reported using a condom all 10 times compared with 75.2% of street sex workers and 67.2% of other sex workers (See figure12).

Figure 12. Frequency of Condom Use



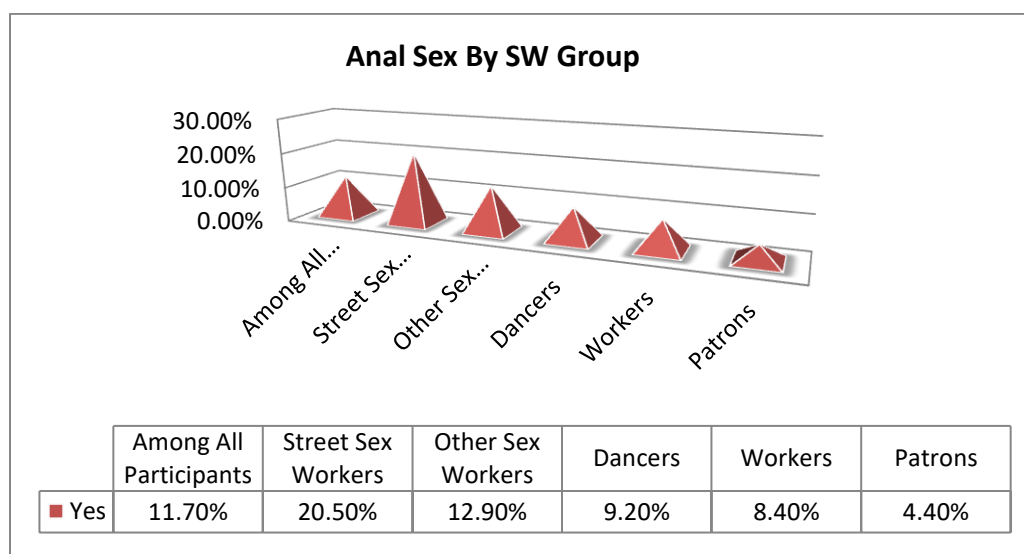
Condom Use with Main Partner

To determine condom use with main partner participants were asked to respond using a confidential self-report card to the question “The last time you had sex with your main partner, did you and your partner use a condom?” Of all participants 35.2% reported using a condom in their last sexual encounter with their main partner. The highest reported use was among other SW (41.7%) followed by street SW (37.9%), dancers (33.9%), patrons (29.4%) and workers (29.1%). (See figure 13).

Figure 13. Self-Reported Condom Use with Main Partner

Types of Sex Engaged in the Past Year

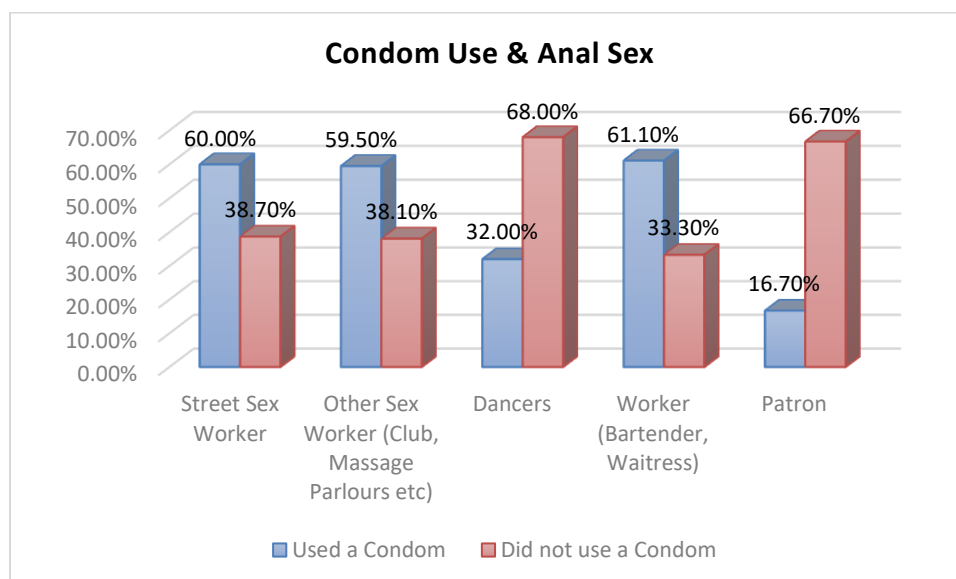
On the types of sex that SW engaged in, 1,124 or 95.8% of the respondents answered the question while 143 (11.7%) of them reported engaging in anal sex, 901 (75.5%) said they had not and 151 (12.6%) said it was not applicable to them. Figure 14 shows Anal Sex practices by SW groups. (See figure 14).

Figure 14. Anal Sex by SW Group

Further analysis of responses from participants who admitted to anal sex in the past year and condom use among groups revealed that approximately two-fifths of street sex workers and other

workers did not use condom in anal sex in the past year and approximately one-third of dancers and two-thirds of patrons did not use condoms in anal sex in the past year (See figure15).

Figure 15. Condom Use & Anal Sex



Condoms Seen

Participants were asked whether they had a condom with them at the time of the interview 32% said they had a condom and presented one to the interviewer while and additional 21% said they had a condom but this was not shown to the interviewer.

Condom Skills in Past 4 Weeks

On condom use skills 71.1% (859) of the respondents said that they had never had a condom slip off during the past 4 weeks, 17.9% (217) said that it has happened once or twice, 7.0% (85) said it had happened 3-5 times, 2.5% (30) said it happened more than 5 times and 1.5% (18) said they didn't remember.

On condoms bursting during the past 4 weeks, 52.4% (634) of the respondents said that they never had a condom burst, 31.2% (8376) said that it has happened once or twice, 9.2% (112) said it had happened 3-5 times and 5.6% (68) said that it had happened more than 5 times and 1.6% (19) said that they didn't remember.

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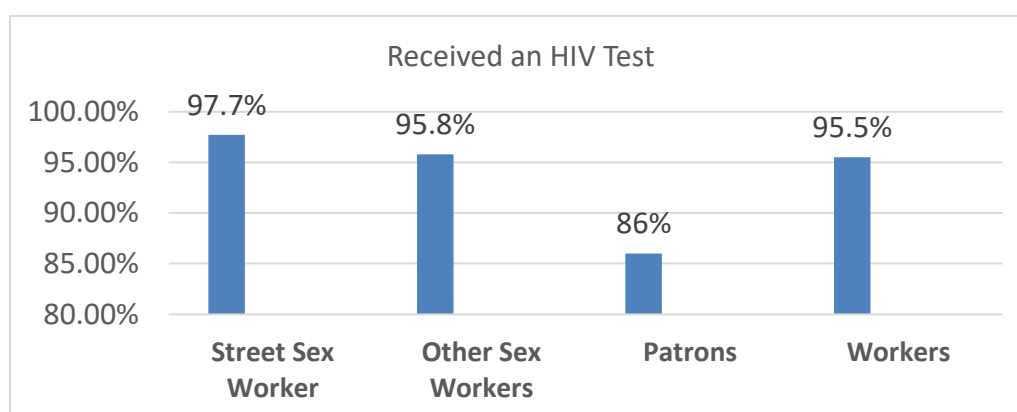
Lubricant Use in Past 4 Weeks

On lubricant use during the past 4 weeks, 20,3% (242) of the respondents said that they used it all the time, 6.9% (82) said they used it almost every time, 26.8% (320) said they used it sometimes, 44.6% (532) said they never used it and .1% (1) said they did not remember. Lubricants were used in 93.3% (529) of respondents for vaginal sex .7% (4) for anal sex and 4.4% (25) for both vaginal and anal sex.

HIV Testing

Knowledge of where to get a HIV test and 'ever been HIV tested' was almost universal among respondents. HIV testing was reported by 94.3% of participants overall having ever received a HIV test (See figure 16).

Figure 16. Ever Received an HIV Test



Analysis by category revealed that 79% overall had been tested in the last twelve months excluding the test conducted during the survey. Exotic dancers were the most frequently tested in the last 12 months with 87.8% having done so while 82.2% of street sex workers, 77.8% of other sex workers, 74% of workers and 70.4% of patrons had also done so in the last 12 months (See table 3).

Table 3. HIV Testing Knowledge and Practices

SW Group	Know Where to get HIV Test	Have Ever Been Tested for HIV	Were Tested for HIV in Past 12 Months
Street Sex Worker	97.00%	97.70%	82.20%
Other Sex Worker (Club, Massage Parlours etc)	96.60%	96.90%	77.80%
Dancer	95.10%	94.80%	87.80%
Worker (Bartender, Waitress)	95.30%	95.40%	74.00%
Patron	94.40%	94.40%	70.80%

HIV Status of Participants

HIV and Syphilis testing revealed that 2% (16) of the 805 sex workers were positive while 3.7% (30) were reactive for syphilis. None of the 254 patrons in the survey tested positive for HIV and 3 or 1% were reactive for syphilis. Meanwhile 1.7% (3) of the 179 workers tested HIV positive while 2 or 1.1% were reactive for syphilis (table 4).

Table 4. HIV & Syphilis Results by Category of Participants

Category	# of Participants	HIV Positive % (N)	Reactive for Syphilis % (N)
Street Sex worker	307	3.2% (6)	6% (18)
Other Sex Worker (Club, Massage Parlours etc.)	267	1.5% (4)	3% (7)
Dancer	231	2.6% (6)	1.7% (4)
Total Sex workers	805	2% (16)	3.7% (30)
Patron	254	0	1% (3)
Worker (Bartender, Waitress)	179	1.7% (3)	1.1% (2)

Adverse Events among Sex Workers

Jailed or Imprisoned: Sex workers reported having experienced a number of adverse events. Spending a night in jail was most commonly reported among sex workers with 40.6% of street sex workers, 34.5% of dancers, 24.1% of other sex workers, 18.8% of workers and 17.1% of patrons having spent a night in jail or in prison (See table 5).

Homelessness: On whether SWs experienced having to sleep outside or in a shelter, 23.4% of street sex workers, 18.0% of other SW, 17.0% of dancers, 11.4% of workers, 8.0% of patrons reported having to do this at some point in their life (See table 5).

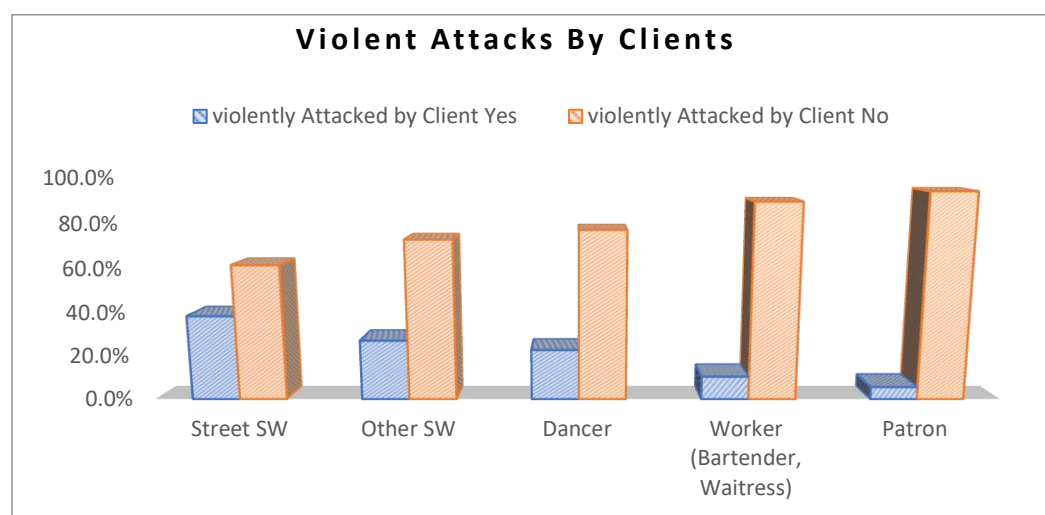
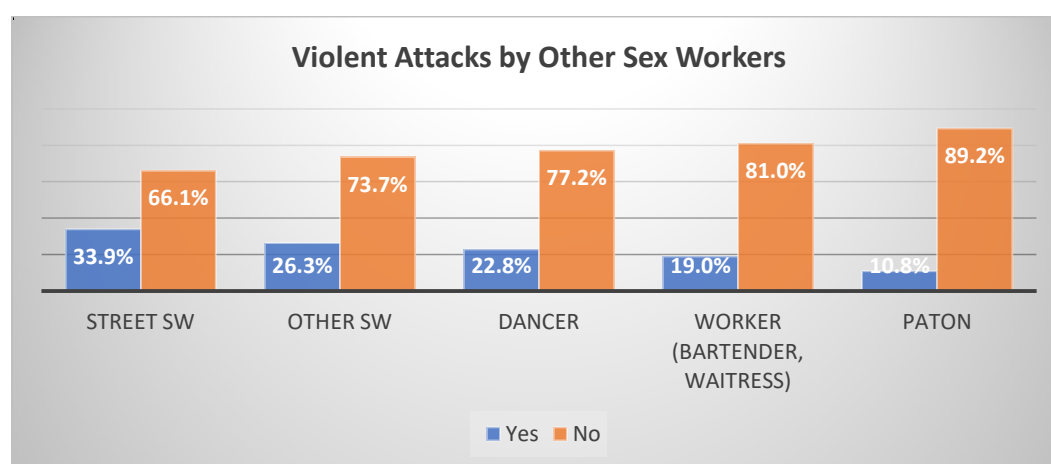
Raped: On the question of having been raped almost one-quarter (22.7%) of SW reported that they had been raped, with street SW and dancers with the highest reported experience of rape

Violently Attacked by a Client: Reported violent attack by clients were highest among street SW (38.5%) followed by other SW (27.2%), dancers (22.9%), workers (10.5%) and patrons (5.6%) (See table 5, figure 17).

Violently Attacked by another SW: A similar pattern of reported attacks were seen that were committed by another SW (See Table 5, figure 18).

Table 5. Adverse Events

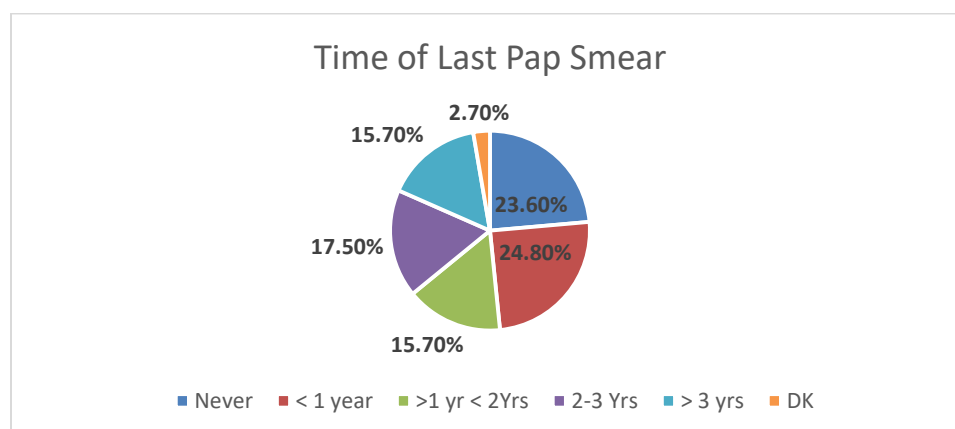
Adverse Event	Street Sex worker	Other Sex worker	Dancer	Worker	Patron
Spent a night in Jail	40.6%	24%	34.5%	18.8%	17.1%
Raped	29.7%	22.3%	29.7%	15.9%	12.9%
Slept outdoors/ homeless	23.4%	18%	17%	11.4%	9%
Violently attacked by a client	38.5%	27.2%	22.9%	10.5%	5.6%

Figure 17. Violent Attacks by Clients**Figure 18. Violent Attacks by Other SW**

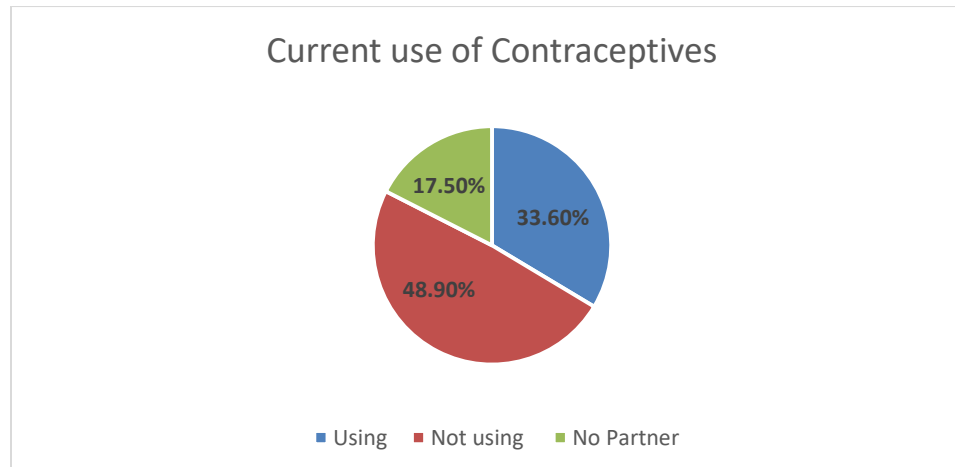
Sexual and Reproductive Health & Family Planning

Pap Smears

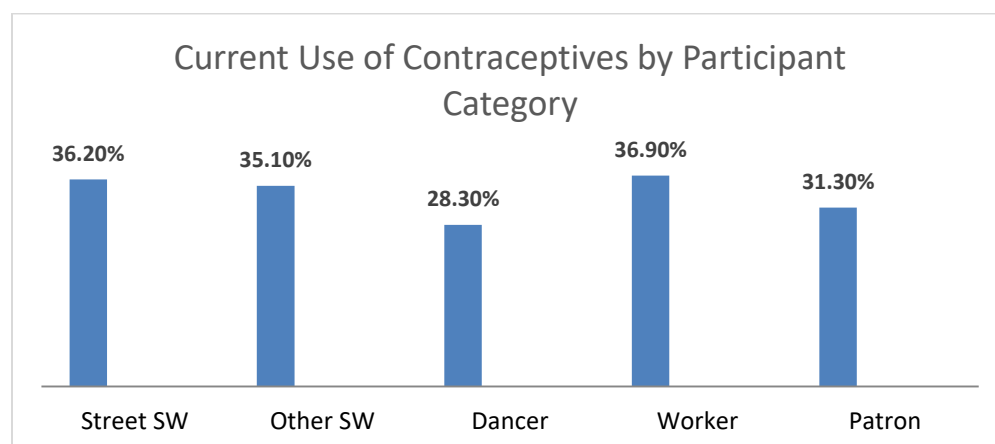
On the question of how long since respondents had their last pap smear, 23.6% reported they had never had one, 24.8% had one within the past year, 15.7% more than one year but less than two years ago, 17.5% more than three years ago and 2.7% did not know when they had their last pap smear (figure 19).

Figure 19. Time of last Pap Smear Test**Contraceptive Use**

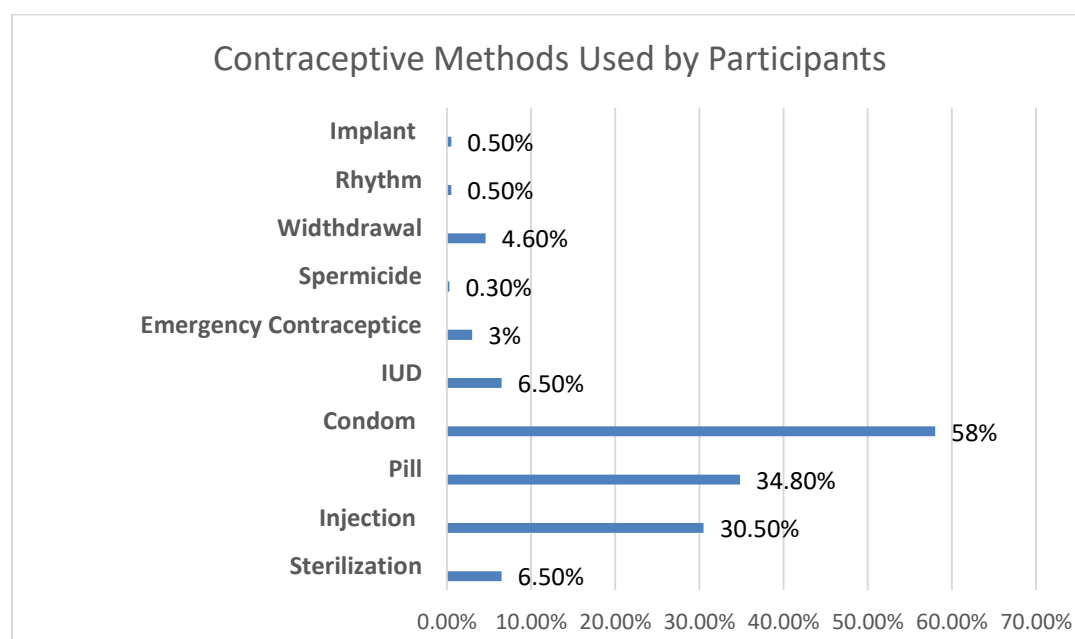
Respondents were asked about their use of contraceptives and 33.6% said that they were using some contraceptive method, 48.9% said that they were not and of 17.5% said that they did not have a partner (figure 20).

Figure 20. Current use of contraceptives

Similar low levels of contraceptive use was found across SW groups with workers reporting the highest contraceptive use (36.9%) and dancers reporting the lowest (28.3%) See figure 21.

Figure 21. Current use of contraceptives by participant category

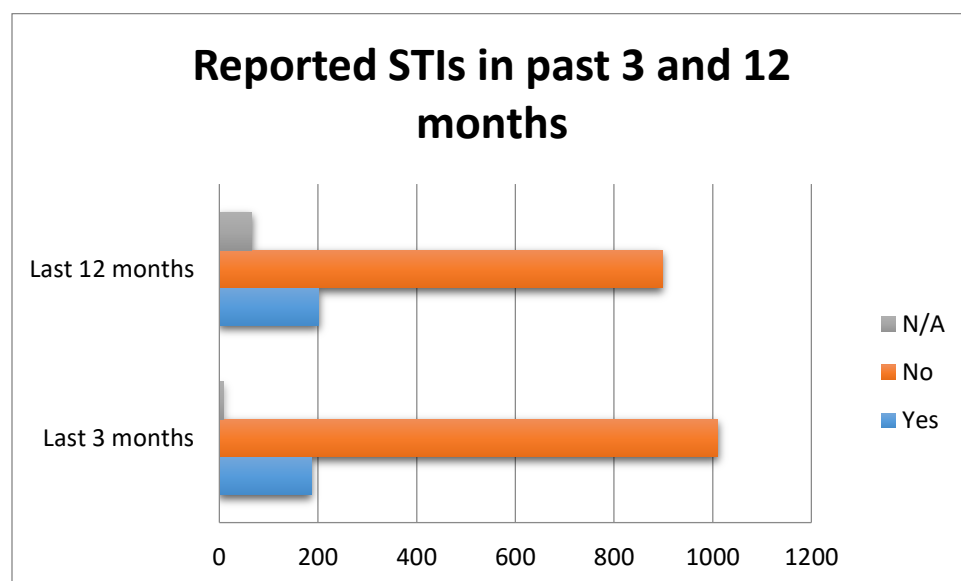
The most reported contraceptive method of choice by respondents was the condom (58.0%) followed by the pill (34.8%) and the injection (30.5%). The least reported methods were sterilization (6.5%), the IUD (3.6%), the withdrawal method (4.6%) and the emergency contraceptive (3.0%) and implant, rhythm and spermicides reported by less than 1% of respondents (figure 22).

Figure 22. Contraceptive methods used by participant category

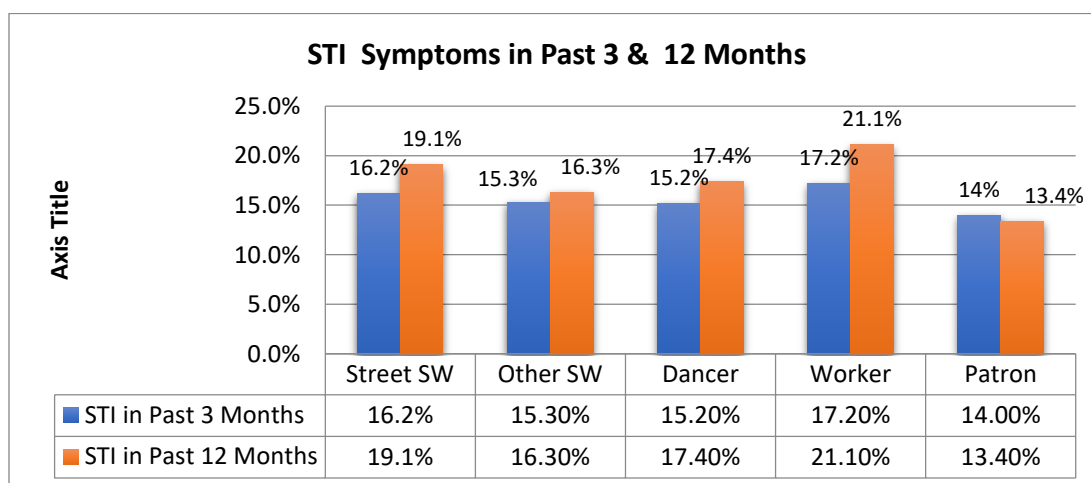
Sexually Transmitted infections and Treatment

To assess reported STIs among respondents they were asked whether or not they had genital discharge, an ulcer, sore or wart or burning on urination in the past 3 months. The majority 1,010 said that they had not had any of these symptoms, while 187 said that they had and 8 said it was not applicable to them. The same question was also asked for those that answered no to 3 months as to whether they had had any of these symptoms over the past 12 months. The majority 898 said that they had not had any of these symptoms, while 201 said that they had and 66 said it was not applicable to them (figure 23).

Figure 23. Reported STIs in past 3 and 12 months



Reported STI over the past 3 and 6 months was analyzed by SW groups. Workers reported the highest incidence of genital discharge, ulcers, sores, warts or burning on urination among all groups (figure 24).

Figure 24. STIs by participant type

Of the persons that said they had an STI symptom in the past 3 months table 6 shows the methods of treatment sought by respondents. The use of a home remedy was highest among street SW (76.9%) and patrons (91.7%) Respondents were also asked if they had sex while they had an STI symptom. Dancers were the most likely to have sex with an STI symptom (81.8%) and street SW were the least likely to have sex with a STI symptom (50.0%). See table 6.

Table 6. Modes of STI Treatment Sought

STI Treatment Mode Sought	Street SW	Other SW	Dancer	Worker	Patron	TOTAL
Treated with Prescription Drugs	53.8%	45.5%	76.5%	71.4%	81.8%	63.90%
Treated at Public Clinic	51.00%	66.70%	37.90%	45.00%	73.70%	53.70%
Treated at Private Doctor	46.40%	55.60%	51.70%	50.00%	50.00%	50.70%
Treated with Home Remedy	76.90%	50.00%	53.30%	55.60%	91.70%	67.30%

On the question of whether or not a health care worker had told respondents in the past 12 months that they had a sexually transmitted infection, 15.9% (192) said that they had, 7.2% (92) said that they had but it was not in the past 12 months and 76.5% (926) said that they had never been told this.

Substance Use

Alcohol was the most reported substance used by respondents in the past 4 weeks with an average of 83.32% having used (See Table 7).

Non-prescription intravenous drug use was not reported by the majority of the respondents, however use was reported by between 1 and 6 people in every group. Among street SW 2.0% (6), other SW 2.3%, (6) and dancers 2.2%, (5), workers 0.60%, (1) and Patron 0.80%, (2) reported its use (See Table 7).

Street sex workers recorded the highest use of crack cocaine among the categories of participants (8%) followed by other sex workers 5.3% and dancers 3.1%. Workers and patrons were the least likely to use crack cocaine (See Table 7).

Marijuana (ganja) was the second most popular substance used by participants after alcohol. Half or more than half of sex workers and exotic dancer used marijuana (figure 28) while 37% of workers and 39.3% of patrons also used that substance (See Table 7).

The use of ecstasy was highest among exotic dancers at 25.6% followed by 20% of street sex workers and 13% of other sex workers. (See Table 7).

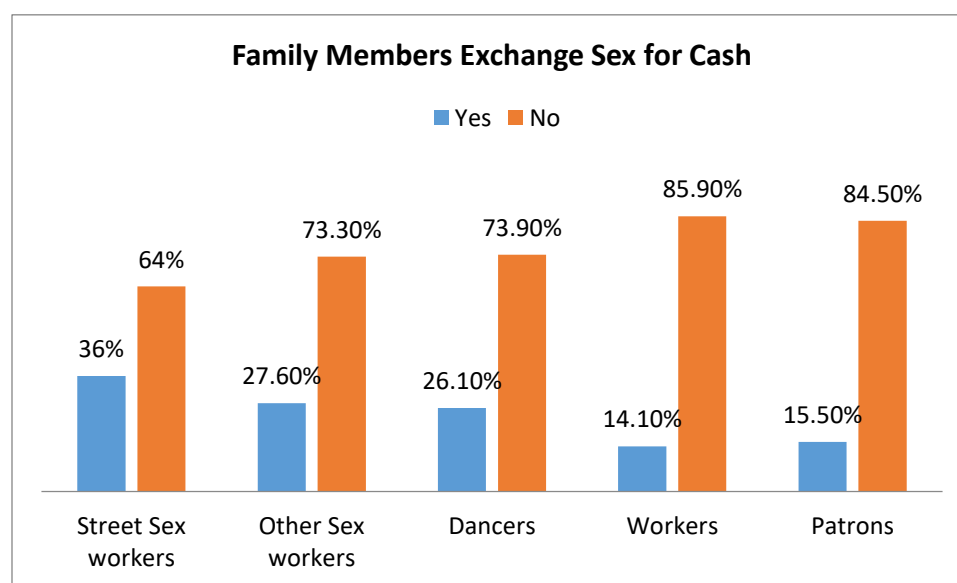
Table 7. Substance Use

Participant	Alcohol Use	IDU Non Prescription Use Past 6 Months	Crack Cocaine Use	Ganja Use	Ecstasy Use
Street Sex Worker	86.20%	2.0%	7.90%	50.40%	20.30%
Other Sex Worker (Club, Massage Parlours etc)	83.70%	2.3%	5.30%	49.10%	13.60%
Dancer	89.50%	2.2%	3.1%	56.60%	25.60%
Worker (Bartender, Waitress)	81.70%	0.60%	1.10%	37.10%	5.30%
Patron	75.50%	0.80%	1.20%	39.30%	4.20%

Family in Sex Work

Participants were asked whether they knew any family member who exchanged sex for cash. On average one third of sex workers and exotic dancers knew a family member who did (See figure 25).

Figure 25. Family Member Exchange Sex for Cash



Exposure to HIV Prevention Activities and Materials

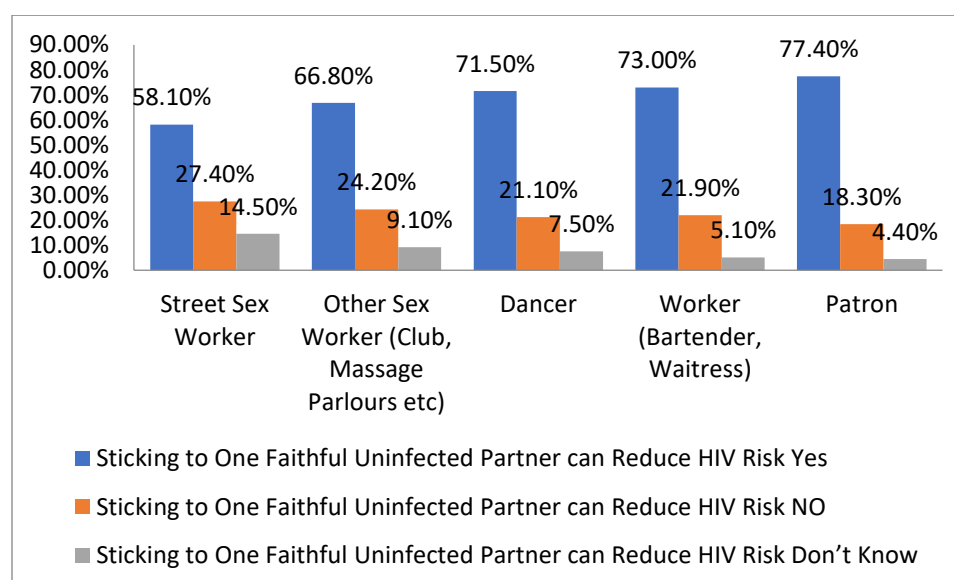
Respondents were asked about their exposure to HIV prevention activities and materials at their work sites. Sex workers and exotic dancers had higher levels of exposure to condom demonstrations, risk reduction conversations and HIV prevention materials than other categories of participants.

Table 8. Exposure to HIV Prevention

Categories	Participated in condom demonstration	Seen HIV prevention material at worksite	Talked with an outreach worker/HIV risk	Talked with an outreach worker/HIV test
Street Sex worker	68.9%	63.7%	72.6%	88.7%
Other Sex worker	71.8%	73.7%	77.4%	84.2%
Dancer	70.2%	72.7%	74.1%	89.5%
Worker	56.2%	66.9%	53.1%	84.3%
Patron	53%	58.2%	45.5%	86.5%

Knowledge and Attitudes to HIV

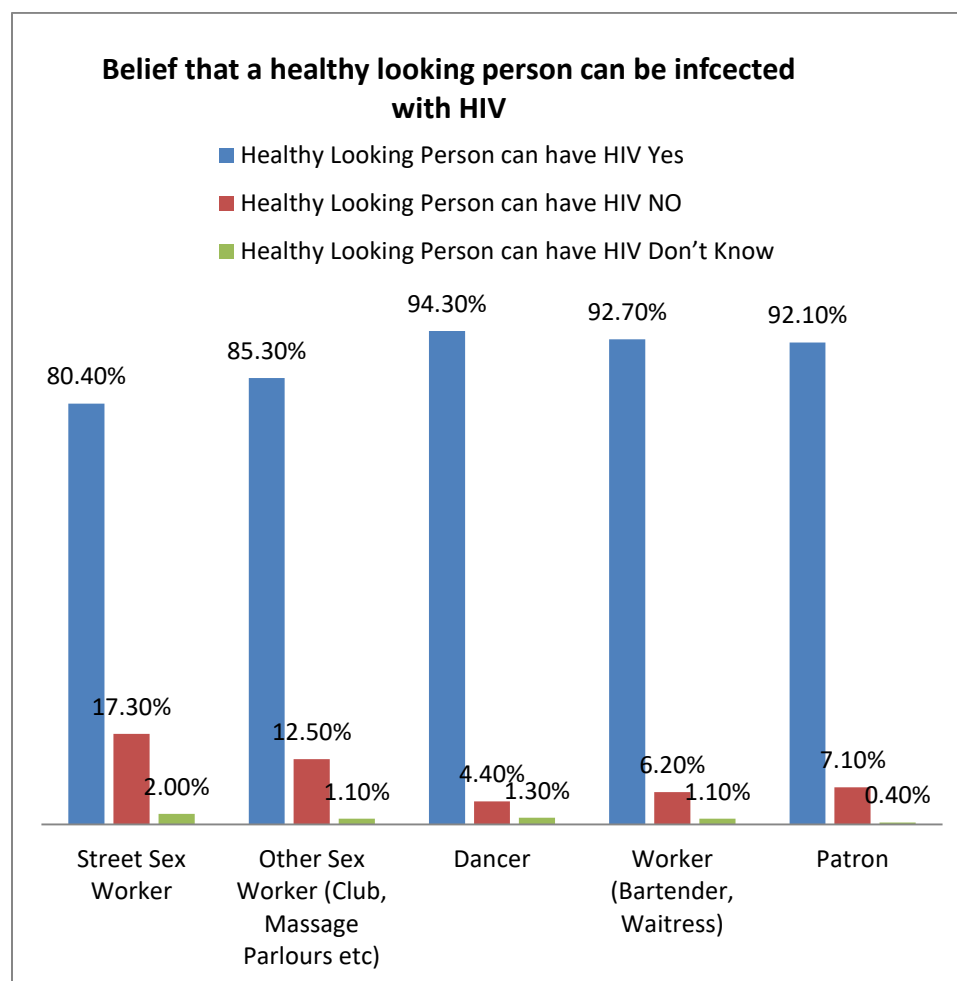
The knowledge, attitudes and perception of HIV risk of the participants were captured by a series of questions. Patrons (77.4%) and workers (73%) endorsed the faithful partner for risk reduction message more than sex workers including exotic dancers. Street sex workers (58.1%) were least likely to endorse the message (See figure 26)

Figure 26. Belief that Sticking to One Partner Reduces Risk of HIV

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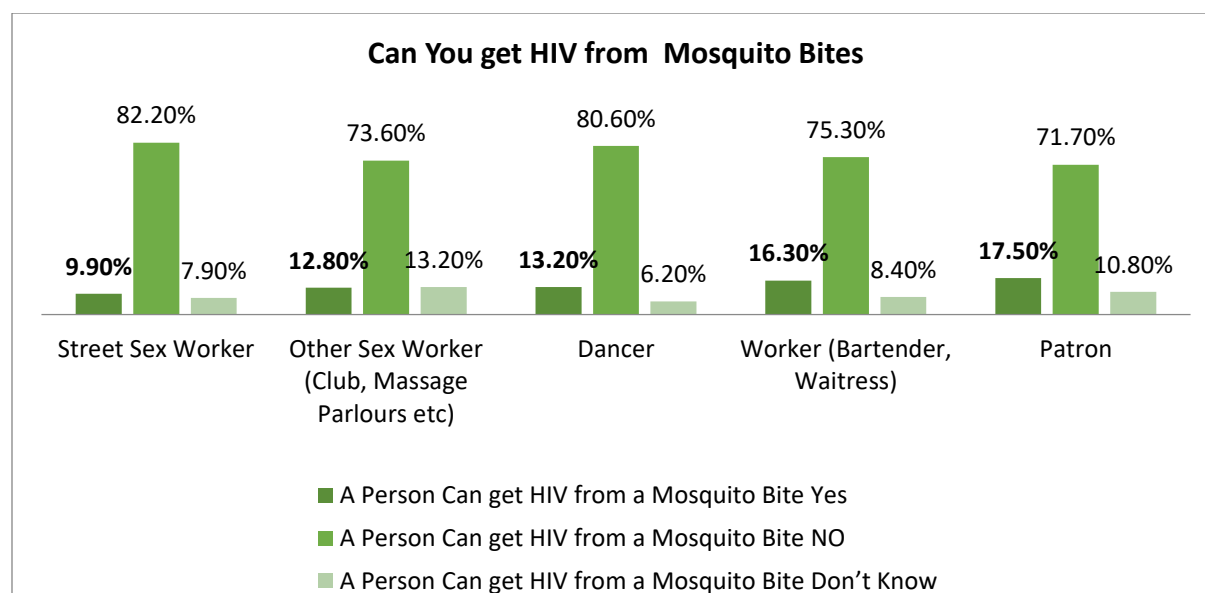
Belief that a healthy looking person can be infected with HIV was generally high but 17.3% of street sex workers and 12.5% of other sex workers did not believe this (See figure 27).

Figure 27. Belief that Healthy Looking Persons Can be infected with HIV



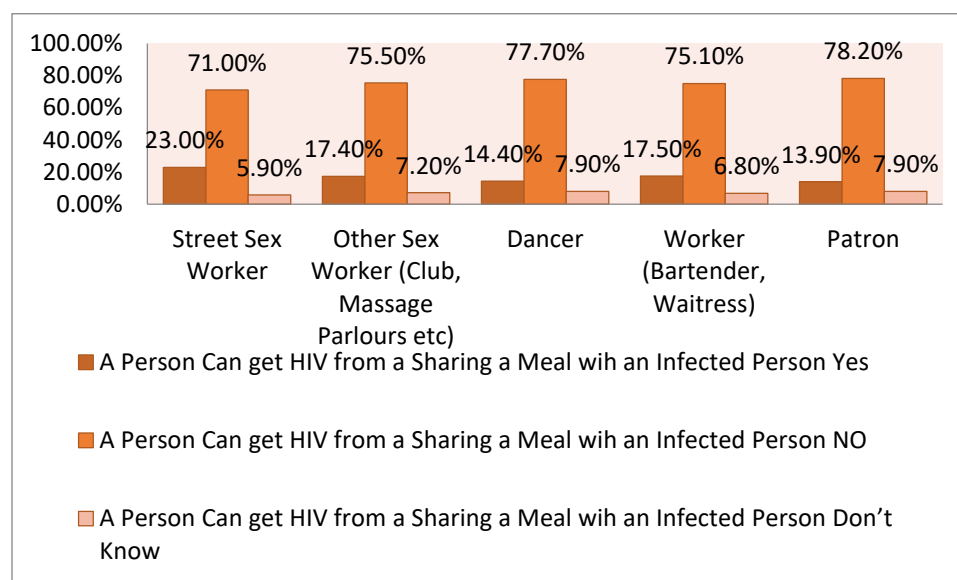
The myth that mosquitoes can transmit HIV was endorsed by between 10% - 17.5% of all categories of participants. Patrons and workers endorsed this myth more than sex workers. Street sex workers (10%) were least likely to believe that mosquitoes can transmit HIV (See figure 28).

Figure 28. Belief that one can get HIV from a Mosquito Bite



An average of 20% of all participants believe that sharing a meal with a HIV infected person is a source of HIV infection (See figure 29).

Figure 29. Belief that one can get HIV by Sharing a meal with an infected Person



On the question “How much risk do you think you have of getting HIV?” table 9 below shows the extent to which sex workers perceive their own risk of contracting HIV. Sex workers and dancers are more likely to perceive themselves at great risk of HIV infection than workers and patrons.

Table 9. HIV Risk Perception

How much risk do you think you have of getting HIV?		Great risk	Moderate risk	Little risk	No risk at all	Does not know
Street Sex Worker	N	177	37	48	20	18
	%	59.0%	12.3%	16.0%	6.7%	6.0%
Other Sex Worker (Club, massage parlour etc.)	N	117	52	56	14	22
	%	44.8%	19.9%	21.5%	5.4%	8.4%
Dancer	N	99	40	50	21	17
	%	43.6%	17.6%	22.0%	9.3%	7.5%
Worker (Bartender, Waitress)	N	29	19	76	34	20
	%	16.3%	10.7%	42.7%	19.1%	11.2%
Patron	N	45	39	88	47	29
	%	18.1%	15.7%	35.5%	19.0%	11.7%
Total	Number	467	187	318	136	106
% of all Participants		38.5%	15.4%	26.2%	11.2%	8.7%

Discussion

The three phases of the fourth generation surveillance of Commercial Sex Workers, Female Patrons and workers at sites where persons meet new sex partners or participate in sexual activity in Jamaica also dubbed the PLACE survey was conducted in order to achieve the following objectives:

- I. To determine prevalence and incidence of HIV and *Syphilis* among high risk women.
- II. To measure proximate & underlying determinants of HIV and STI transmission
- III. To monitor program coverage
- IV. To establish closer contacts with key populations in order to expand prevention and treatment services
- V. To determine effectiveness of prevention interventions among FSW
- VI. To estimate female sex workers population size
- VII. To understand social and sexual networks of FSW

This section will discuss the implications of the results outlined in the previous section in relation to these objectives.

The population of female sex workers recorded in the site verification phase indicated a busiest time population of 1,258 FSWs, 980 exotic dancers and 858 PWs. No record was made of the number of patrons. These women exchanged sex for cash at street sex work sites, exotic clubs, bars, massage parlours, outdoor hang-out spots, night clubs and malls. Most street sex workers operated at a street sex work site while most other sex workers operated at an exotic club with dancers. Most of the non- street based sex workers operated from exotic clubs followed by massage parlours. These numbers give an indication of the penetration of sex work across Jamaica. St Catherine, St Andrew, St Ann, Westmoreland, Hanover and St James had the largest concentration of sex workers but every parish even the three not included in this 2017 study has its own sex work population that are aligned to the sites listed above but also to other sites and constant surveillance is necessary to keep track of changes.

The 1238 participants in this study consisted of 589 sex workers, 216 exotic dancers, 254 patrons and 179 place workers. A third of these women were in a union at the time of the study. One fifth had previously been in a union. Seventy percent had a main male sex partner while 16% had a

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female main sex partner. It is important that discussion of risk and condom use behaviours be mindful of these variations in union status and include condom negotiation with both paying and non- paying partners.

HIV testing among participants has established that the steady decline in the prevalence of HIV among sex workers continues, moving from 4.9% in 2008 to 4.1% in 2011, and 2.8% in 2014 and in this 2017 surveillance to 2%. Syphilis testing also indicated a decline in syphilis with 3 % of sex workers and 1% of patrons being reactive in this 2017 surveillance compared to 6.3% in 2014. These results are very rewarding given the high level of resources put into reaching sex workers. They also reflect some effectiveness of the interventions for FSWs. However, it should be noted that the sex workers reached and tested in this survey could exclude certain types or categories of sex workers who are not captured in the radar both of this study and general targeted interventions.

For reasons of affordability and timeliness the decision was taken to only include sites with large numbers of female sex workers soliciting in this as well as the previous two surveys (2011 & 2014). It is clear based on the high access to testing by participants (between 70% - 82% having a HIV test in the last 12 months) and exposure to HIV interventions (over 80% by sex workers and dancers) that many of these participants represent the low hanging fruit. Qualitative research indicates that there are sex workers who solicit from other sites including the community or do so seasonally. These women are more likely to provide services without a condom or not be tested for HIV and other STIs. It is important that regular outreach for sex workers seek to identify and target these women as well as those in the traditional sites included in this survey.

There are a number of results in this study that indicate the proximate & underlying determinants of HIV and STI transmission. The main proximate determinant noted in this survey was the inconsistent use of condoms with main partners and regular paying clients. This is likely manifested in between 16%-19% of Street SWs, other SWs and dancers reporting STI symptoms in the last 12 months even when condom use with non- regular paying clients among these groups is reported at between 91%-97%. Partner/ relationship intimacy is clearly the biggest threat to consistent condom use. Ways must be explored to reach the regular client and main

partners of sex workers as well as to continue to emphasize this aspect in the risk reduction conversations with the FSWs.

Incomplete education and early child bearing were the main socio-economic drivers and underlying determinants of entry into sex work and increased risk of HIV/STI infection. It is of note that the median age of sexual initiation (16 yrs.) of the participants was similar to that recorded for in the national reproductive health survey (10). Despite this similarity less than 50% of participants had completed high school and dependency on cash for sex as the main source of income was high among participants where even 16% of workers and 13% of patrons who did not classify themselves as sex workers admitted to cash for sex as their main source of income. The vulnerability created by poverty and lack of social support was also manifested in the adverse events recorded among participants. For example 40.6% of street sex workers reported having spent a night in jail, 23.4% had been homeless and 23% had experienced rape while almost 40% had been attacked by a client. Interventions to reduce socio-economic vulnerability among FSWs should be strengthened and may require partnerships and structural approaches that go beyond the remit of the HIV/STI Programme and the Ministry of Health.

The participants particularly the FSW ways of coping with these physical and psychological conditions of sex work is manifested in the recorded common use of alcohol (83.32%) and drugs (> 50% marijuana use among SWs) which in themselves create risk through disinhibition. Discussions with sex workers about the ways in which ingestion of alcohol and performance enhancing substances can be reduced in order to mitigate risk must be included in the interventions delivered to this target population.

Programme coverage was reflected in the high levels of exposure to HIV prevention messages and materials recorded by participants. However it is important to note that while 68.9% of street SW, 71.8% of other SW and 70.2% of dancers had participated in condom demonstration only 56.2% of workers and 53% of patrons had. Similarly while over 70% of SWs and dancers had talked with an outreach worker about their HIV risk only 53% of workers and 45% of patrons had done so. Access to HIV testing was the most evenly provided aspect of the HIV programme as while almost 88% of exotic dancers had received an HIV test in the last 12 months 70% of

patrons had also done so. Since workers and patrons at sites where sex workers operate also exchange sex for money they need to receive similar interventions as the sex workers.

Declines in HIV and syphilis prevalence has been listed earlier as one of the indicators of the effectiveness of prevention interventions among FSW so also are the knowledge and attitudes towards HIV. Knowledge of where to get an HIV test was almost universal among the participants (97%) but endorsement of some myths still remain too high. For example between 13% -17 % of sex workers still believe that a healthy looking person cannot have HIV and between 13% -23% would not share a meal with someone who is HIV positive. Appropriate treatment seeking behaviours may also indicate programme effectiveness and the results indicate a mixed bag. Street and other sex workers less frequently reported having sex with an STI symptom than dancers, workers and patrons and also reported less use of prescription drugs however they reported more frequent use of home remedy and approximately 50% sought treatment for STIs at both private and public clinics.

Recommendations

- It is vital the successes achieved among the female sex workers that have been reached be maintained through adequate allocation of resources for prevention interventions with this population. It is clear that the patrons and workers who are also associated with the sites where sex workers and exotic dancers solicit are also involved in transactional sex. Similar levels of intervention should be provided to these women.
- Ways must be found to adequately address the threat partner intimacy to condom use. It is possible to reach the regular client and main partners of sex workers in order to have them assess their own risk of HIV/STI infection. This is necessary as the women are more likely to use condoms with these partners if the men are open to or request condom use.
- The use of alcohol and drugs must be addressed consistently particularly the use of ecstasy among exotic dancers and crack cocaine among street sex workers. The slight trend towards the use of IV drugs needs to be monitored closely and appropriate interventions be provided pro-actively.

- The underlying determinants and vulnerability of female sex workers need to be addressed to prevent risk taking in times of desperation and make the reduction in number of partners be possible particularly among street based sex workers. Their heavy reliance on sex work for the main source of income and more adverse life events put them at greater risk. Ways must be found to provide more social support for those most at risk by creating linkages to government and non- governmental social agencies.
- Increase STI education in order to promote appropriate treatment seeking behaviours among all sex workers as well as the workers and patrons.

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APPENDIX 1

TITLE: The 4th (2017) Generation Surveillance of Commercial Sex Workers Female Patrons and Workers of Sites where Persons Meet Sex Partners or Participate in Sexual Activity in Jamaica

3. DATE: August 2017

4. INVESTIGATORS:

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1. INTRODUCTION

The proposed bio-behavioral surveillance survey is 4th (2017) generation surveillance of female sex workers (FSW) female patrons (FP) and PLACE workers (PW) at sites where persons meet sex partners or participate in sexual activity in exchange for money in Jamaica. The survey follows on from the 2011 and 2014 bio-behavioral surveillance surveys continuing to track HIV prevalence among these groups of females and collect information that will guide and improve HIV/STI interventions among FSW, PW and FP at risk. This protocol describes the PLACE approach that will guide the execution of the 2017 bio-behavioral surveillance survey (BBSS) among FSW, other PW and FP. The PLACE methodology has been proven effective in achieving a high quality sample that facilitates the monitoring of trends of the epidemic in key population of FWS over time.

2. JUSTIFICATION

Sex workers (SWs) and men who have sex with men are the key populations at the highest risk of HIV infection due to their specific social and behavioural characteristics. In the countries like Jamaica with both low-level and concentrated HIV epidemics, these populations account for most HIV infections and are also called “core groups” for HIV transmission.

The 2011 and 2014 bio-behavioral surveillance of female sex workers were approved by the MOH Ethics Committee. The current protocol reflects only minor changes to the 2011 PLACE survey, in line with the current focus on FSW, FP and PW and the need to gather current key population data to measure the behavioural impact of HIV/STI interventions. Data that emerged from the 2011 and 2014 survey showed FSW and PW are 10-11 times more likely, to be diagnosed with HIV when compared to male patrons. This pattern was consistent when data for Syphilis and Gonorrhea was examined and therefore men have been excluded from the 2017 surveillance (Ministry of Health, 2012).

5. **SITE OF RESEARCH:** Cross-sectional bio-behavioral surveys of people at venues identified as places where people meet new sexual partner in order to estimate prevalence of infection among this population, risk factors associated with infection and gaps in STI prevention programs.
6. **NUMBER OF PARTICIPANTS TO BE ENROLLED:** Approximately 2,880 individuals will be interviewed in a minimum of two study areas. These individuals will be community informants, female sex workers (FSW) other PLACE workers (PW) and female patrons

(FP). Approximately 1,140 individuals will be interviewed from the South East health region, 875 from the Western region and another 875 from both the North East and Southern regions (See table 1). The target sample size will provide sufficient power in data analysis to identify and describe the proximate factors associated with risky sexual behavior and HIV/STIs prevalence in the particular sample of participants. The allocation of FSW and FP interviews across areas is based on the expected size of the FSW and FP populations in each of the regions.

Table 10: Proposed distribution of participants by regions

Participants	Priority Prevention Areas (PPAs)				Gender	Age
	South East	North East/ Southern	Western	Total		
Community Informants	400	400	400	1200	M&F	18+
Venue Informants (1 per venue)	340	170	170	680	M&F	18+
Female Sex Workers (FSW)	250	175	175	600	F	16+
Female Patrons (FP)	80	70	70	210	F	16+
PLACE Workers (PW)	70	60	60	190	F	16+
Total	1140	875	875	2880		

7. **PROPOSED DURATION OF THE STUDY:** The study will take approximately eight (8) months to complete. The data collection component entails three sets of interviews, they are: A. community informant, B. site verification and C. interviews with workers and patrons. The first phase of data collection will commence with community informants. Approximately 5-10 minutes will be scheduled per interview. Site

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verification interviews will follow community interviews and about 20 minutes per interview is expected. The final set of interviews will take place with sex workers and female workers and female patrons. The estimated time per interview in the final stage of data collection will be about 30-40 minutes.

8. SUMMARY

The aim of this protocol is to describe the PLACE approach that will guide the execution of the 2017 bio-behavioral surveillance survey (BBSS) among FSW, other PW and FP at sites that they frequent. The guidelines and procedures that will be followed in the 2017 PLACE Survey for conducting interviews with persons at increased risk of acquiring and transmitting infections as well as HIV and Syphilis are the same as those that were approved in the 2011 PLACE Survey. HIV and STI testing, pre and post-test counseling and provision of test results to participants; ensuring confidentiality and quality control remain fundamental ethical requirements and safeguards of this study. These guidelines and procedures have been further refined and published in PLACE in the Caribbean; Institutionalizing PLACE methodology to reach key populations in the Caribbean (Scott & Byfield, 2013).

The survey will establish HIV prevalence among these groups of females and collect information that will guide and improve HIV/STIs interventions among FSW, PW and FP at risk. Similar to the 2011 protocol STI testing that requires urine samples will be conducted. This was absent from the 2014 surveillance. The four specific sexual transmittants: *Trichomonas vaginalis* (trichomonas), *Chlamydia trachomatis* (Chlamydia) *Neisseria gonorrhoeae* (GC) and *Mycoplasma genitalium* will be tested using a multiplex PCR test that will detect the four pathogens simultaneously.

The specific objectives of the study are:

- To determine prevalence and incidence of HIV and *Syphilis* among high risk women
To establish prevalence levels of 4 common sexually transmitted infections:- *Trichomonas*, *Chlamydia*, *Gonorrhoea* and *Mycoplasma genitalium* among FSW, PW and FP.
- To measure proximate & underlying determinants of HIV and STI transmission
- To monitor program coverage
- To establish closer contacts with key populations in order to expand prevention and treatment services
- To determine effectiveness of prevention interventions among FSW
- To estimate female sex workers population size
- To understand social and sexual networks of FSW

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7. BACKGROUND

Various data sources have shown that there are a number of factors, which if unattended, have the potential to erode the gains made in HIV/STI prevention and control efforts at the national level. These include lack of consistent condom use among sex workers, especially with their regular clients and main partners. Furthermore, some of these behaviors are driven by increasing poverty, population dynamics, and inadequate academic certification among SW among other variables. The PLACE (2012) surveillance report established that over 88% of sex workers did not complete secondary education. Most of the women had children (64%-78.1%) and were below the age of 30 (51.7%), with 1 in every 3 having the experience of spending a night in jail. Additionally, many reported that they had no employment (28.1%-48.5%) or were only employed part-time (29.1%). These realities will likely exacerbate the negative impact of general socio-economic challenges on the population of sex workers and further increase risky sexual practices. The current 2017 bio-behavioral surveillance data is critical in informing policies and programs to better respond to the sexual health risks of sex workers.

8. LITERATURE REVIEW

There are a number of success stories to report on in the National response to HIV. The country was recently among 20 countries in the world and specifically three in Caribbean to cut HIV incidence by more than 50% between 2001 and 2012. In addition, fewer persons are currently contracting HIV (new HIV cases declined by 42% as of 2013 according to UNAIDS 2014 estimates) than in previous years and concomitantly, more persons infected with HIV/AIDS are being provided with better access to treatment, in particular access to Antiretroviral medication (ARVs) and HIV testing operations have probably never been more robust and strategic. Additionally, the up-coming 2017 bio-behavioural surveillance survey among FSW and FP will be the sixth since 1990 and is one of the main data sources used in the design and implementation of HIV prevention and control programs, targeting sex workers as well as in the monitoring and evaluation of the said programs.

However, further work is required in order reduce sexual transmission of HIV in the general population and in particular among key sub-populations including sex workers and men who have sex with men (MSM) as demonstrated by country and regional specific data. For example, the Caribbean—based on the adult HIV prevalence rate of

1.1% rate as of 2013—is the second most affected region in the world with respect to the HIV/AIDS epidemic. Jamaica, like the Dominican Republic and more so Haiti, is disproportionately affected by HIV as its prevalence rate of 1.8 exceeds that of the region (UNAIDS, 2013).

It is estimated that 30,265 persons of the Jamaican adult population, as of 2013, are infected with HIV and approximately half are not aware that they are infected. Higher HIV prevalence has been recorded in vulnerable groups such as sex workers (SW) (5%), MSM (32%), and persons with STIs (3.6%), crack/cocaine users (5%), and prison inmates (3.3%). Sentinel surveillance of women attending antenatal clinics indicate that there is no significant change in the prevalence of HIV over the last decade despite prevention efforts, which stands at 1% (2013).

Research has shown that the odds of FSW contracting HIV is about 13.5 times greater than other women (Baran et al., as cited in UNAIDS, 2013). Global HIV prevalence rate among sex workers was estimated to be about 12%. Despite the HIV rate among sex workers in Jamaica being lower than that of the global rate and also the marginal decline in HIV prevalence rate among sex workers, down from 4.9% in 2008 to 4.1% in 2011, about 1 in every 10 new HIV cases in 2012 was estimated to have been associated with commercial sex. Furthermore, 20.4% of adults with HIV reported sex with commercial sex workers over the period 1982-December 2010 (Ministry of Health, 2012). The population of sex workers has grown from about 14,957 (UNAIDS, 2013) in 2011 to 18,696 as of 2014 (Global AIDS Progress Report, 2014), an increase of 3,739 additional workers over the period. Moreover, over 38%-41% of sex workers who participated in the 2011 PLACE Survey had no HIV test before the survey while 30.5%-39% knew that they had at least one STI.

9. METHODOLOGY

Research Questions

The study will explore the following questions:

- What are the characteristics of venues where people meet new sexual partners in Jamaica?
- Where are venues located?
- What evidence is there of HIV prevention program efforts at the venues?
- Are condoms available onsite?

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- Are HIV prevention messages displayed onsite?
- What is the prevalence and incidence of HIV and STIs among women at venues identified as places where people meet new sexual partners?
- What is the prevalence and incidence of HIV and STIs among women who exchange sex for money?
- What are the socio-demographic and behavioral characteristics of these groups?
- What is the extent to which these groups have access to HIV prevention programs?
- What are the characteristics of those infected with HIV?
- What are the characteristics of those infected with other STIs?

Rationale

There is an urgent need in Jamaica to improve current prevention programs to limit the spread of HIV. Many approaches are needed to contain the epidemic including school-based programs, mass media campaigns and targeted approaches for those most at risk. Prevention efforts require current information about the HIV and STIs epidemics in high risk groups.

Participants:

- 1200 Community Informants aged 18 and older will be asked to identify venues
- 680 Venue Informants will be asked about the characteristics of a venue that he or she is knowledgeable about (1 informant per venue)
- 600 Female Sex workers (FSW)
- 400 'Other PLACE' workers (PW) and Female patrons (FP)

Participants will be recruited from the four health regions. The sample size was calculated in order to have separate precise estimates categories of participants (i.e., FSW Other PW and FP) and to have a large enough sample to allow comparison with previous surveys. The number of community informants is based on the expectation that 400 interviews in each area will identify over 90% of the eligible venues. This reflects experience with previous surveys in Jamaica.

Inclusion criteria and how identified and recruited:

- Community Informants: Community informants (n=1200) male and females ages 18 and older who are knowledgeable about where people go to meet new sexual partners will be recruited by convenience from places where target population members are likely to congregate or visit.
- Female Sex Workers (FSW): Women (n=600) who report exchanging sex for money in the past 12 months. FSW will be identified and recruited by study team members or peers

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through peer networks or at places where sex workers frequent for the purpose of selling sex.

- Female PLACE Workers (PW)): Women (n=190) who work at venues identified by community informants as places where people go to meet new sexual partners. They will be recruited by study team members.
- Female Patrons (FP): Female patrons (n=210) in attendance at PLACE venues. They will be recruited by study team members using random sampling methods to obtain a representative sample of patrons from the venue. The method will be adapted to fit the type of venue. For example, in closed environments such as a bar or club, an imaginary X will be drawn by the study team across the floor of the venue and persons standing near the X will be approached for an interview. This approach has been implemented in previous patron surveys including the 2011 PLACE Survey.
- Adolescent patrons Persons ages 16 or 17 at the venue that are not accompanying adult relatives or friends nor on a family errand will be included and are the exception to the following exclusion criteria.
- **Exclusion criteria:**
 - Persons younger than age 18 that are only present at the site because they are accompanying adult relatives or friends or are on an errand will be excluded from participating. This criteria was also applied to previous PLACE surveys.

Study Design:

Cross-sectional bio-behavioral surveys of women at venues identified as places where people meet new sexual partners in order to estimate prevalence of infection among this population, risk factors associated with infection and gaps in STIs prevention programs.

Study Procedures

Stakeholder Meetings: One stakeholder meeting in each of the 4 health regions will be convened to explain the purpose of the surveillance to the community, identify types of people who could be approached as community informants, specify target numbers of each type to interview, and generate a preliminary list of venues. Stakeholders will include peer educators and outreach workers.

1. **Community Informant Interviews:** Following this meeting, brief interviews with 1200 community informants will be conducted. Interviewers will ask each informant a few questions about himself or herself (See questions in appendices under the subsection Form

A) and to name venues where people meet new sexual partners and where each venue is located. No personal identifiers will be obtained from the informant. Community informants in previous surveys in Jamaica have included taxi drivers, people who work in bars and clubs, peer educators, and others. The list of venues named and their addresses will be compiled in an Excel file and sorted by geographic area. The number of informants reporting each venue will be tallied and the list collapsed into a comprehensive list of unique venues.

2. **Venue Visits:** Trained interviewers will visit each reported venue (up to a maximum of 680 venues) and interview a respondent at each venue (Form B) about the characteristics of the venue. Form B will include questions about whether there are workers and patrons at the venue and how many (if any) of the workers and patrons are likely to be SWs. The respondent may be a manager, a worker, or someone else who is knowledgeable about the venue. No (personally) identifying information will be obtained from the respondent. GPS coordinates will be recorded for each venue. Data from Form B will be entered and tabulated.
3. **Sampling Venues for Interviews with patrons, sex workers and other PLACE workers:** The study team will review the venue data from Form B to determine which venues to visit for bio-behavioral interviews with sex worker and Other PLACE workers and patrons based on statistical sampling methods as well as pragmatic considerations such as how many people of the target group are likely to be reached at the venue and the likely willingness of the people to participate. Questions on Form C will be posed to these participants. The Study Lead will use the existing interventions/workshops and outreach activities to sensitize and to encourage participation. The recruitment process, experiences and questionnaires used at venues will be monitored and documented by supervisors using Form E. The survey and testing will be conducted at times arranged with the participants at the selected venues or locations identified as comfortable and safe.
4. **Screening for Recruitment into sex worker, other PLACE worker and patrons:** There are no specific criteria that must be met in order to participate in the initial screening. During the screening, if the person does not meet the eligibility criteria or if targets for that type of participants have already been reached, the interview will be concluded. Screening Questionnaire is Form C and it includes obtaining informed consent.
5. **Eligible Persons Invited to participate in survey and testing:** Eligible persons will be invited to participate in the survey and testing. Only persons who agree to survey and testing for HIV, *Syphilis*, *Trichomonas vaginalis* (trichomonas), *Chlamydia trachomatis* (Chlamydia) *Neisseria gonorrhoeae* (GC) and *Mycoplasma genitalium* will be invited to

participate. At the end of the interview a phone card worth JD\$500 will be given to participants.

6. **Informed Consent:** People will be asked to give verbal consent once they are identified as eligible to participate in the survey. Venue workers, all sex workers, and patrons will be asked to consent to both the interview and testing. Individuals will be advised of their right to refuse to participate or to end participation at any time.

7. Content of Surveys:

- The survey will administer three separate questionnaires (Forms A-C), plus an additional confidential answer sheet (Form D) for sensitive questions posed on Form C. Form A will be administered to Community informants and will contain questions on their age, occupation, what parish they reside in and the name(s) and location of places that they know of where SW solicit clients, and where people go to meet new sexual partners.
- Form B will be used in the site verification and will be administered to a knowledgeable person at each of the venues selected for verification. The instrument will capture data about the number of people attending the venue, their characteristics, and about the environment of the venue (e.g. is alcohol served).
- The questions on Form C will be administered to sex workers, patrons and staff and will include questions about sexual behavior, signs of STI, exposure to HIV prevention programs, and knowledge of HIV. Responses for the most sensitive questions about sexual behavior and condom use will be documented by participants themselves on a confidential card (Form D). The interviewer will read aloud the questions to participants, if required and respondents will privately record the answers. This has proven to address participants' concern of confidentiality, desire for privacy and promote validity of responses.
- Each of these questionnaires (A-D) had been approved by the ethical review committee at the Ministry of Health for previous surveillance studies, including the 2011 and 2014 PLACE surveys.

8. **Rapid Testing and Provision of Results:** Both rapid HIV and *Syphilis* tests and the collection of urine samples for four specific sexual transmittants: *Trichomonas vaginalis* (trichomonas), *Chlamydia trachomatis* (Chlamydia) *Neisseria gonorrhoeae* (GC) and *Mycoplasma genitalium* will be conducted at sites by trained practitioners. The procedures which will be used to conduct the tests will adhere to the outreach testing algorithm

protocols for HIV and *Syphilis* approved by the National Public Health Lab (NPHL) (See attached). The Determine test device will be used to conduct HIV testing. All four (4) STIs will be tested using a multiplex PCR test that will detect the four pathogens simultaneously. The Standard SD Bioline 3.0 rapid method will be used to test for Syphilis. These devices will be labeled using a unique identifier code generated from participants' information (comprising of part of the participant's birth date and mother's last name). Each of the tests will be accompanied by the required pre and post-test counseling, informed oral consent, standard safety and operating procedures and quality control measures. Counseling will be conducted in enclosed and private areas. Persons whose test results are negative will be provided with risk reduction counseling. Those, whose results are reactive or inconclusive, in accordance with the NPHL protocols, will be counseled and referred to the Contact Investigator at the nearest public Health Facility or a private clinician of their choice for confirmatory testing including CD4 count and anti-retroviral treatment. Class 1 notification form will be completed and delivered to the Parish Medical Officer of Health at the Parish Health Department for persons who tested positive for HIV, and Syphilis.

Persons whose test results are negative will be provided with risk reduction counseling. Those, whose results are reactive or inconclusive, in accordance with the NPHL protocols, will be counseled and referred to the Contact Investigator at the nearest public Health Facility or a private clinician of their choice for confirmatory testing including CD4 count and anti-retroviral treatment. Class 1 notification form will be completed and delivered to the Parish Medical Officer of Health at the Parish Health Department for persons who tested positive for HIV, and/ or and Syphilis.

9. **Methods of recruiting:** All participants will be recruited by experienced, well-supervised interviewers.
 - a. **Community Informants:** Community informants will be recruited by trained interviewers from public locations in each study area in Jamaica. Interviewers will be provided with a list of individuals to interview with target numbers and types (e.g. taxi drivers, security guards, market sellers) by the Study Lead. Potential community informants will be approached by a trained interviewer who explains the purpose of the study and requests verbal informed consent. The interview and request for informed consent will be conducted in English and Patios, as appropriate. The respondent's name or other identifying information will not be asked or recorded.

used by stakeholders such as the National HIV/STI Program of the Ministry of Health and the NFPB-SHA to design HIV prevention interventions for sex workers, high risk women and for the general population. The key persons at the Ministry of Health and NFPB-SHA who will use the data are the Senior Medical Officer HIV/STI/Tb Unit; Director Health Promotion and Protection and the Adolescent Health Programme Manager HIV/STI/Tb Unit, Dr Nicola Shyers, Ms Andrea Campbell and Mrs. Marion Scott- Thomas, respectively to improve programs in the area of HIV and STIs prevention and control.

- 13. Full description of risks and measures to minimize risks:** There is minimal risk of breach of confidentiality from this study. Confidentiality of subjects will be maintained and protected by the use of study numbers on study specimens, case report forms and other study forms. Study documents including consent forms (Form I in appendix) with the subjects' names, will be stored in a separate logbook in a locked cabinet at the NFPB-SHA. Syphilis and HIV are reportable diseases in Jamaica. All HIV and Syphilis results will be entered into the MOH surveillance system using the standard system to maintain confidentiality of patient results. To ensure confidentiality and minimize risks to participants, all HIV, Syphilis and STI testing will be conducted by staff already employed to the National HIV/STI Programme or the Regional Health Authorities who conduct outreach VCT for HIV and Syphilis as part of their routine function.

Community informants will be asked to name public places where they believe persons visit to meet new sexual partners. Providing this information may cause feelings of embarrassment and the study provides persons with the option of refusing to participate. No one can be put at risk of legal prosecution because no identifying information will be recorded.

Knowledgeable people asked about the venues may feel embarrassed to report that people meet new sexual partners at the venue, or that commercial sex workers seek clients at the venue. There is no reason for them to fear official retaliation against the venue because local owners and manager as well as local officials including the police will have endorsed the assessment. There have been no reprisals against sites in any of the over 30 completed implementations of PLACE assessments.

Participants may feel embarrassed to report their socio-demographic characteristics or sexual or other risky behavior. However, protocols were developed to minimize the feelings of embarrassment or reluctance that a person may have in answering the questions. Participation is brief, voluntary and anonymous. Anyone may refuse to answer any question at any time. The purpose of sensitive questions is described to allay respondent's fears. The most sensitive questions will be asked using self-

completed answer sheets. This proposed procedure has been used for previous surveys in Jamaica with approval from the Advisory Panel on Ethics and Medical Legal Affairs Team of the Ministry of Health. No breaches of confidentiality have been previously reported.

14. **Data monitoring and analysis:** Approximately 2,880 individuals will be interviewed in the following areas: community informant, site verification and participant interview (including sex workers, other PLACE workers and female patrons). The target sample sizes will provide sufficient power in data analysis to describe the sexual behavior of people with a high rate of new sexual partnerships. The data collected are quantitative and will be analyzed by generating frequencies and cross-tabulations for a final report. Data may be stratified by age (16-24, 25+), risk types, educational attainment, regions, among other variables.
15. **Identifiers in research data:** The only place that a participant's ID number and the name will be stored together is in a Study Log that is only available to the Co-Principal Investigators and the Study Lead. Information from the log will also be confidentially entered into the national surveillance data base for persons infected with HIV or Syphilis. This process has been ongoing in Jamaica for over 30 years and there has been no known breach of confidentiality.
16. **Confidentiality of the data.** Confidentiality will be discussed in detail with field workers. Each field worker will be required to sign a pledge of confidentiality (See form G in appendix). Confidentiality issues will be addressed during a training session on research ethics. The PowerPoint Presentation used in ethics training for interviewers in previous PLACE studies will be used at the time of interviewer training.

Most of the venues are public venues where anyone is welcome to socialize. A few private venues may sometimes be included in the study. The list of names and addresses of venues will not be shared with anyone outside of the study team.

The characteristics of venues obtained from knowledgeable people at the venues will not be shared outside of the study team except in reports summarizing characteristics across all venues. For example, the report might state that 'knowledgeable people at 50% of venues reported that beer is consumed onsite' but a list of venues by name where beer is consumed will not be provided. Similarly, characteristics of patrons socializing at these venues will be presented in summary form by age group and regions, and not by site.

Maps of the venues will be shared with the intervention team and published but not in a manner that would identify illegal activities or permit easy identification of venues. The large size of the dots also masks exactly where a venue is located. Without landmarks or the name of the neighborhood, it is difficult for those not on the study team to recognize the

study area. The purpose of the mapping is to show clustering and the extent of risk behavior, not to identify particular venues so that they can be located. Consequently, the base maps used to ground the points collected by GPS are invariably incomplete and distort the true location of venues.

Three types of maps are provided to the intervention team and also published in the report:

- (1) a map of the venues showing the number of times a community informant reported the venues;
- (2) a map of the venues showing whether condoms and other AIDS prevention programs are available onsite or not; and
- (3) a map identifying priority venues for HIV/AIDS prevention programs. These maps may take into account the presence of commercial sex, patronage and alcohol consumption but would not be labeled in a way that would identify that any location was the venue for onsite illegal activity. Venues for commercial sex socializing would not be identified on any map in any report or publication.

Paper copies of completed questionnaires will be kept in a locked cabinet at the NFPB-SHA office. Only the Principal Investigators and the Study Lead will have access to the cabinet. The data will be stored on a password-secured hard drive of the data entry computer and saved on flash drives will be kept in the locked cabinet.

Data security for storage and transmission

For electronic data stored on a desk top computer: Secure network and password access

For portable computing devices/external storage devices (e.g. laptop computer, PDA, CDs, memory sticks): Power-on password

For hardcopy data (including human biological specimens, CDs, tapes, etc.): Locked suite or office and Locked cabinet

Post-study disposition of identifiable data or human biological materials

No blood samples will be stored as part of the 2017 PLACE Protocol. The Study log will not be destroyed. After the study is completed and the results published, it will be kept by the Principal Investigators in a secure and locked place, awaiting use in a follow-up study. The soft copy of the data set will be owned by the M&E Units of the National Family Planning Board-SHA and the National HIV/STI Programme. Any related publication must be with permission of the Principal Investigators.

Informed Consent: The interviewer will request verbal, anonymous informed consent after the purpose of the study is briefly described. The interview and request for informed consent will be conducted in English and Patois.

Workers and patrons will be asked to sign an informed consent statement. They will be allowed to sign their initials if they are concerned about signing their name. They will be consent separately to interview and HIV testing.

Justification for a waiver of *written* (i.e., signed) consent: The only record linking the subject and the research would be the consent document and the principal risk would be potential harm resulting from a breach of confidentiality (e.g., study topic is sensitive so that public knowledge of participation could be damaging). *Participants will be asked whether they want documentation linking them with the research and the participants' wishes will govern whether they sign the form.*

The only place that the names of the participants are recorded will be the study log. We do not want to have any other form with the name of the participants due to the potential harm resulting from a breach of confidentiality.

Persons whose HIV or Syphilis test results are found to be presumptively positive will be appropriately counseled and the necessary referral will be made for treatment including CD4 count and anti-retroviral treatment, where indicated. The log will be kept under lock and key under the care of the Study Lead with only the Principal Investigators having back up access. No names will be used in the analysis or reporting of the data.

Justification for a waiver of *parental* (i.e., signed) consent for participants age 16 and 17

In line with the previous waiver received in the 2011 and 2014 PLACE studies, we are requesting the waiver of seeking parental consent for participants who are age 16 and 17. In Jamaica, age 16 is the age of consent for sex. We have questions in the questionnaire to determine whether a person age 16 or 17 is at a venue as a child member of a family or as an adult. We ask if they are at the site on an errand or with parents. If they are on a family errand or with parents, they are not eligible. We do not want to ask for parental consent from other individuals, age 16-17 who are at the site because they are at the site as an adult, and because asking for information to find the parents and obtain their consent could potentially harm the participant. Individuals between ages 16-17 who participate in this study will benefit from the study by getting tested for infections and getting treated. We do not want to prevent people age 16-17 from participating.

This study involves only minimal risk. The survey will pose questions to participants about sexual behavior and condom use. They will be required to provide a blood sample and the 4th (2017) Generation Surveillance of Commercial Sex Workers Female Patrons and Workers of Sites where Persons Meet Sex Partners or Participate in Sexual Activity in Jamaica

sample will be obtained using sterile lancet. A new lancet will be used for each of the participants. The research will not adversely affect subjects. Access to the names of participants will be restricted to only two officers who will be legally bound to adhere to the data confidentiality and management requirements of the survey. These officers will be the Study Lead and Principal Investigators.

Where will the subjects be studied? Communities within each health region of Jamaica will be identified for the study during the stakeholder meetings. Most interviews will be conducted in public locations in quiet, private areas within a public location. A few private venues may sometimes be included in the study.

Privacy: Prior to the implementation of the study, experienced interviewers will be trained on survey techniques and confidentiality issues associated with this study. They will be instructed in procedures for keeping all parts of the interview confidential.

For the participant, efforts will be made to ensure the interview takes place in a private setting for the purposes of confidentiality and to reduce any potential stigma. Interviews will, to the extent possible, be conducted in venues where people socialize, though in some cases the respondent may request to move to a different location within the venue in order to preserve privacy (e.g. a room set aside by the venue manager, an unoccupied corner of the venue, outside the venue, a private residence). Upon arrival at the venue, the interviewers will identify a suitable location where privacy can be maximized, checking to be sure the participants' responses cannot be overheard by others at the venue. If a suitable location cannot be found within the venue, interviewers will explore other locations outside and nearby the venue. All females present at sites may be considered for participation if they meet the eligibility criteria.

Incentive for participation: Study Participants will be given a telephone card with the value of up to JD\$500 as an incentive for participation. Those who get results and are infected will be appropriately counseled and treated. Those who are HIV positive will be referred for appropriate management including CD4 count and anti-retroviral treatment where indicated. Where the referrals are to private doctors, the study will pay the initial visit costs. The main inducement to participate is to get tested for HIV and Syphilis.

Costs to be borne by subjects: The only cost to participants is time.

Use of Existing Data, Records or Human Biological Specimens: Previous surveillance studies conducted by the Ministry of Health have included surveillance of the same study population. Information on test results from previous surveillance surveys has been collected. It is possible that some of the same people who were tested previously will be

tested again. We will ask persons if they have been tested previously. If they have been tested previously, we will ask them for permission to look up their previous results in order to compare them with the new test results.

Funding: This study is being funded by the Ministry of Health with project funds received from the Global Fund.

Declaration of Conformity with Guidelines: All Principles enunciated in the **Ministry of Health's Guidelines for the Conduct of Research on Human Subjects** have been complied with.

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APPENDICES:**FORM A: COMMUNITY INFORMANT INTERVIEW – ONE PER INFORMANT**

No.	Questions	Coding categories
	Hello. I am working on a study which is being conducted by the Ministry of Health in Jamaica to identify where better health programs are needed. We want to talk to persons like you in the community who we believe can assist us. The purpose of the study is to find out where people go to meet new sexual partners. Please tell us the names and locations of places where you think people meet new sexual partners. We don't want to know the names of any private residences. We are just interested in public places. If you tell us where these places are, we will visit them to see if they want to have a health program there. Telling us the names and locations of these public places should take between 5 and 10 minutes. We will not ask your name or any information about yourself. You will not be contacted in the future and your answers cannot be linked back to you. Your participation is completely voluntary and you may decline to answer any specific question or completely refuse to participate. We would greatly appreciate your help in responding to these questions.	
	<u>ASK EVERYONE:</u> Could you tell me a few places or events where people go to meet new sex partners in <this community>? USE SPACE BELOW TO LIST PLACES. THEN PROBE: • Are there places where people can pay for sex services, including oral sex, lap dances and full-body massage? • Are there streets where sex workers solicit clients? • Are there other social events where people who meet each other may have a one night or brief sexual relationship? • Are there websites or chat rooms available to find someone for sex?	
	<u>Record if the informant agree or refuse to participant.</u> 1. <u>Agree</u> 2. <u>Refused</u>	

LIST OF SITES AND EVENTS

FOR EACH SITE OR EVENT LISTED HERE, FILL OUT A SITE/EVENT REPORT FORM. AFTER FORMS ARE COMPLETED, THANK THE RESPONDENT FOR THEIR TIME.

1. _____

2. _____

3. _____

4. _____

5. _____

6. _____

7. _____

8. _____

FOR EACH PLACE NAMED BY COMMUNITY INFORMANTS – FILL IN A1-A8.

PLACE REPORT Interviewer Number: _____ Sequential CI Number: _____ Report: 1 2 3 4 5 6 7 8					
A1	Record the name of the site as given by community informant : _____				
A2	Circle the parish in which interview is being conducted	Kingston 1 St Andrew 2 St Catherine 3 St. Mary 4	St Ann 5 St Elizabeth 6 Portland 7	St Thomas 8 Westmoreland 9 Hanover 10	Clarendon 11 Manchester 12 St James 13 Trelawny 14
A3	What is the name of the community in which the site is located?	Name: _____ Census Code: _____			
A4	Select the category which best describes the place/site	Bar 1 Night Club 2 Exotic Club 3 Massage Parlor 4 Fast food restaurant 5	Internet (Specify type below) 6 Outdoor Site (Specify type below) 7 Event (Specify type below) 8 Other (Specify type below) 9		
A5	Specify type. _____ _____				
A6	Key populations at place?	i) Female sex workers? 1 YES 2 NO ii) MSM? 1 YES 2 NO			
A7	Specify in detail the address of the site/how to find it.				

A8	Record the date of Interview: Date of interview ____/____/____ <i>Day/Month/Year</i>
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FORM B: SITE AND EVENT QUESTIONNAIRE

B1 PLACE ID Site Number: _____

SITE AND EVENT QUESTIONNAIRE

B1 PLACE ID Site Number: _____

No.	Questions	Coding Categories
B2-B4	B2 Interviewer Number: _____ B3 Initials: _____ B4 Date (DD/MM/YY): ____/____/____	
B5	Parish Of Place Circle 1 KINGSTON 1 ST ANDREW 2 ST CATHERINE 3 ST. MARY 4 ST ANN 5 ST ELIZABETH 6 PORTLAND 7 ST THOMAS 8 WESTMORELAND 9 HANOVER 10 CLARENDON 11 MANCHESTER 12 ST JAMES 13 TRELAWNY 14	
B6a	Name and Type of PLACE from CI Reports:	
B6b	Actual Name Of Place:	
B7	Actual Community Where Site/Event Is Located:	
B8	Correct Address / Directions	
B9	Type Of Place BAR 1 NIGHT CLUB 2 EXOTIC CLUB / SEX SHOP 3 FAST FOOD / RESTAURANT / INTERNET CAFE 4 OUTDOOR HANG OUT SPOT / PARK/ SW SITE 5 HOTEL/ MOTEL /GUESTHOUSE 10 CAMPUS SITE 11 MALL/ PLAZA 12 INTERNET SITE 13 DAY OF WEEK EVENT OR DANCE 16 OTHER INDOOR SITE 86 OTHER INDOOR EVENT 87 OTHER OUTDOOR SITE 88 OTHER OUTDOOR EVENT 89	
B10	Describe Type Of Site/Event In Words.	

No.	Questions	Coding Categories
B11	Role Of Respondent At Site	STAFF 1 SECURITY GUARD 2 PATRON 3 VENDOR IN AREA 4 SEX WORKER 5 STUDENT 6 B11A OTHER (SPECIFY) : _____ 7
B12	Interviewer Opinion: How knowledgeable is this respondent likely to be about the activities and patrons at the site?	EXTREMELY KNOWLEDGEABLE 1 KNOWLEDGEABLE 2 NOT VERY KNOWLEDGEABLE 3
Hello. I am working on a study approved by the Ministry of Health in Jamaica. We want to talk to people like you who know about this community. The purpose of the study is to identify where better health programs are needed to prevent the spread of diseases that are transmitted by sex. I would like to ask you some questions about activities that occur at this place, the people who come here, and programs that may take place here. The interview should take between 30-40 minutes of your time. I won't ask your name or any other identifying information. Your participation is completely voluntary and you may decline to answer any question or completely refuse to participate. We appreciate your help responding to these questions, even though we cannot financially compensate you for your time. You may not personally benefit directly from the study, but the results may be used to plan a new health program for this area. This is what we will do with the information you give us. Your answers will be recorded on a paper that only identifies this place with a number. Your name will not be recorded anywhere and we won't ask any personal information about you except to make sure you are age 18 or older. We are asking people these questions at more than one hundred places in this area. The questionnaires will be kept in a locked cabinet. The only people who will see the questionnaires are people working on this study. An ethical review board has reviewed this study. If you have any questions please feel free to contact Dr. Sharlene Jarrett, National Family Planning Board-Sexual Health Agency, 968-1627.		
B13	Questions To Respondent Begin Here: ARE YOU WILLING TO ANSWER THESE QUESTIONS? If Respondent Refuses, Find Another.	YES 1 NO 2

No.	Questions	Coding Categories
B14a B14b	Was An Interview Completed? If Not Why Not?	YES 1 NO BECAUSE: NO WILLING RESPONDENT 2 ALL POTENTIAL RESPONDENTS TOO YOUNG 3 SITE CLOSED TEMPORARILY 4 SITE CLOSED PERMANENTLY OR NO LONGER A SITE 5 OTHER(SPECIFY REASON): _____ 8
B15	HOW MANY YEARS HAS THIS SITE BEEN IN OPERATION?	< 1 YEAR 1 1-2 YEARS 2 3-5 YEARS 3 MORE THAN 5 YEARS 4 NOT APPLICABLE 9
B16a- B16h	WHICH TYPES OF ACTIVITIES TAKE PLACE HERE? Circle Yes Or No For Each.	YES NO A ALCOHOL (BEER OR TONIC WINE) CONSUMED 1 2 B ECSTASY USED ONSITE 1 2 C CRACK/COCAINE USED ONSITE 1 2 D EXOTIC OR GO-GO DANCING 1 2 E SEX ON STAGE 1 2 F MUSIC (LIVE OR RECORDED) 1 2 G MASSAGE 1 2 H PEOPLE WAITING FOR A BUS OR TAXI 1 2
B17a- B1b	I HAVE BEEN TOLD THAT PEOPLE MEET SEXUAL PARTNERS AT PLACES LIKE THIS. DO.....Read List.	YES NO DK A DO MEN MEET NEW FEMALE SEXUAL PARTNERS HERE? 1 2 8 B DO WOMEN MEET NEW MALE SEX PARTNERS HERE? 1 2 8 C DO MEN COME HERE TO BUY SEX FROM WOMEN? 1 2 8 D DOES SOMEONE ONSITE HELP PARTNERS HOOK UP? 1 2 8 E DO FEMALE SEX WORKERS COME HERE TO FIND CUSTOMERS? 1 2 8

No.	Questions	Coding Categories
B17c- B17j	Continued.... I HAVE BEEN TOLD THAT PEOPLE MEET SEXUAL PARTNERS AT PLACES LIKE THIS. DO..... Read List.	YES NO DK F DO ANY MALE STAFF MEET NEW SEXUAL PARTNERS HERE? 1 2 8 G DO ANY FEMALE STAFF MEET NEW SEXUAL PARTNERS HERE? 1 2 8 H CAN PEOPLE HAVE SEX ONSITE? 1 2 8 I DO GAY MEN MEET MALE SEX PARTNERS HERE? 1 2 8 J DO GAY MEN WHO ARE SEX WORKERS COME HERE TO FIND CUSTOMERS? 1 2 8
B18	WHEN DO FEMALE SEX WORKERS COME HERE?	EVERY DAY /NIGHT 1 NONE/ NOT AT ALL 2 SOME DAYS/NIGHTS 3
B19	DOES THIS PLACE HAVE SPECIAL DAYS OR NIGHTS?	NO SPECIAL DAYS OR NIGHTS: 0 YES, SPECIAL TIMES (specify below) 1
B20	A NAME OF SPECIAL: _____ B HOW OFTEN: _____ TIME: _____AM/PM TO _____AM/PM	
B21	A NAME OF SPECIAL: _____ B HOW OFTEN: _____ TIME: _____AM/PM TO _____AM/PM	

No.	Questions	Coding Categories																		
B22a- B22e	A WHAT IS THE BUSIEST DAY OF THE WEEK HERE? Circle 1. B_C WHAT ARE THE BUSIEST HOURS ON THAT DAY? D_E DURING THAT TIME, HOW MANY MEN AND WOMEN DO YOU EXPECT TO BE SOCIALIZING AT THIS SITE? Read Options.	A : MONDAY TUESDAY WEDNESDAY THURSDAY FRIDAY SATURDAY SUNDAY B: BUSY HOURS BEGIN: ____ ____ AM/ PM C HOURS END: ____ ____ AM /PM D MEN: E WOMEN : <table> <tr> <td>FEWER THAN 25</td><td>1</td><td>1</td></tr> <tr> <td>25-50</td><td>2</td><td>2</td></tr> <tr> <td>51-100</td><td>3</td><td>3</td></tr> <tr> <td>101-150</td><td>4</td><td>4</td></tr> <tr> <td>151-200</td><td>5</td><td>5</td></tr> <tr> <td>MORE THAN 200</td><td>6</td><td>6</td></tr> </table>	FEWER THAN 25	1	1	25-50	2	2	51-100	3	3	101-150	4	4	151-200	5	5	MORE THAN 200	6	6
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51-100	3	3																		
101-150	4	4																		
151-200	5	5																		
MORE THAN 200	6	6																		
B23a- B23j	WHAT TYPES OF WORKERS ARE AT THIS SITE? If There Are Zero Workers Of A Certain Type, Write Zero "0".	YES NO A. ANY FEMALE DANCERS? 1 2 B. IF YES: NUMBER OF FEMALE DANCERS ON A BUSY NIGHT: _____ C. ANY MALE DANCERS? 1 2 D. IF YES: NUMBER OF MALE DANCERS ON A BUSY NIGHT: _____ E. ANY PAID STAFF EXCLUDING DANCERS? 1 2 F. IF YES NUMBER OF MALE STAFF EXCLUDING DANCERS: _____ G. IF YES NUMBER OF FEMALE STAFF EXCLUDING DANCERS: _____ _____ H. ON A BUSY NIGHT ANY SEX WORKERS ON SITE? 1 2 IF YES HOW MANY SEX WORKERS? I. HOW MANY FEMALE SEX WORKERS (EXCLUDING DANCERS) _____ _____ J. HOW MANY MALE SEX WORKERS (EXCLUDING DANCERS)? _____ _____																		

No.	Questions	Coding Categories																																																				
B24a- B24d	IN THE PAST 12 MONTHS (SINCE PUT IN DATE), HAVE THERE BEEN ANY HIV PREVENTION ACTIVITIES AT THIS SITE? HAS THERE BEEN.....Ask Each.	<table border="1"> <thead> <tr> <th></th><th>YES</th><th>NO</th><th>DK</th></tr> </thead> <tbody> <tr> <td>A. ANY HIV/AIDS PREVENTION?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>B. ANY CONDOM PROMOTION?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>C ANY PERSONS ACTUALLY TESTED ONSITE FOR HIV?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>D ANY SAFER SEX EDUCATION BY OUTREACH WORKERS?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>E. ANY OUTREACH THROUGH SEX WORK PEER EDUCATORS?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>F. ANY OUTREACH THROUGH INFLUENTIAL STAFF?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>G. ANY OUTREACH OR PREVENTION BY MOH?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>H. ANY OUTREACH OR PREVENTION BY JAS?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>I. ANY OUTREACH OR PREVENTION BY RED CROSS?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>J. ANY OUTREACH BY ANY OTHER ORGANIZATION?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>K. IF YES: WHAT ORGANIZATION: _____</td><td></td><td></td><td></td></tr> <tr> <td>L. ANY OTHER SURVEYS ABOUT HIV PREVENTION AT SITE?</td><td>1</td><td>2</td><td>9</td></tr> </tbody> </table>		YES	NO	DK	A. ANY HIV/AIDS PREVENTION?	1	2	9	B. ANY CONDOM PROMOTION?	1	2	9	C ANY PERSONS ACTUALLY TESTED ONSITE FOR HIV?	1	2	9	D ANY SAFER SEX EDUCATION BY OUTREACH WORKERS?	1	2	9	E. ANY OUTREACH THROUGH SEX WORK PEER EDUCATORS?	1	2	9	F. ANY OUTREACH THROUGH INFLUENTIAL STAFF?	1	2	9	G. ANY OUTREACH OR PREVENTION BY MOH?	1	2	9	H. ANY OUTREACH OR PREVENTION BY JAS?	1	2	9	I. ANY OUTREACH OR PREVENTION BY RED CROSS?	1	2	9	J. ANY OUTREACH BY ANY OTHER ORGANIZATION?	1	2	9	K. IF YES: WHAT ORGANIZATION: _____				L. ANY OTHER SURVEYS ABOUT HIV PREVENTION AT SITE?	1	2	9
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L. ANY OTHER SURVEYS ABOUT HIV PREVENTION AT SITE?	1	2	9																																																			
B25	IN THE PAST 12 MONTHS, HOW OFTEN HAVE CONDOMS BEEN AVAILABLE HERE?	<table border="1"> <thead> <tr> <th></th><th>ALWAYS</th><th>SOMETIMES</th><th>NEVER</th><th>DON'T KNOW</th></tr> </thead> <tbody> <tr> <td></td><td>1</td><td>2</td><td>3</td><td>8</td></tr> </tbody> </table>		ALWAYS	SOMETIMES	NEVER	DON'T KNOW		1	2	3	8																																										
	ALWAYS	SOMETIMES	NEVER	DON'T KNOW																																																		
	1	2	3	8																																																		
B26	CAN I GET A CONDOM HERE NOW? IF YES, CAN I SEE ONE?	<table border="1"> <thead> <tr> <th></th><th>YES CONDOMS HERE BUT YOU CANT SEE ONE</th><th>YES CONDOMS HERE AND A CONDOM WAS SEEN</th><th>NO CONDOMS HERE NOW</th></tr> </thead> <tbody> <tr> <td></td><td>1</td><td>2</td><td>3</td></tr> </tbody> </table>		YES CONDOMS HERE BUT YOU CANT SEE ONE	YES CONDOMS HERE AND A CONDOM WAS SEEN	NO CONDOMS HERE NOW		1	2	3																																												
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B27a B27b B27c B27d	Interviewer Observation: Evidence Of HIV/AIDS Prevention Activities Noted By Interviewer At The Site	<table border="1"> <thead> <tr> <th></th><th>YES</th><th>NO</th></tr> </thead> <tbody> <tr> <td>A ANY HIV/AIDS POSTERS DISPLAYED?</td><td>1</td><td>2</td></tr> <tr> <td>B ANY CONDOM PROMOTION OR USE POSTERS?</td><td>1</td><td>2</td></tr> <tr> <td>C. CONDOMS VISIBLE?</td><td>1</td><td>2</td></tr> <tr> <td>D CONDOMS FOR SALE ON SITE?</td><td>1</td><td>2</td></tr> </tbody> </table>		YES	NO	A ANY HIV/AIDS POSTERS DISPLAYED?	1	2	B ANY CONDOM PROMOTION OR USE POSTERS?	1	2	C. CONDOMS VISIBLE?	1	2	D CONDOMS FOR SALE ON SITE?	1	2																																					
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D CONDOMS FOR SALE ON SITE?	1	2																																																				
B28	WOULD YOU LIKE TO GIVE CONTACT INFORMATION IN CASE WE WANT TO DO SOME OUTREACH ACTIVITIES HERE?	<table border="1"> <thead> <tr> <th></th><th>DECLINED TO GIVE CONTACT INFORMATION</th></tr> </thead> <tbody> <tr> <td></td><td>_____</td></tr> <tr> <td>NAME: _____ MOBILE: _____</td><td></td></tr> </tbody> </table>		DECLINED TO GIVE CONTACT INFORMATION		_____	NAME: _____ MOBILE: _____																																															
	DECLINED TO GIVE CONTACT INFORMATION																																																					

NAME: _____ MOBILE: _____																																																						

No.	Questions	Coding Categories
B29	Interviewer Opinion How Supportive Are Gatekeepers Regarding Safe Sex Practices At The Site?	VERY SUPPORTIVE 1 SUPPORTIVE 2 NOT SUPPORTIVE 3 VERY UNSUPPORTIVE 4 DO NOT KNOW 5 NO GATEKEEPERS 9
B30	Interviewer's Prevention Suggestions for Place:	

Form C

QUESTIONNAIRE #: _____ PLACE SITE ID #: _____

No.	INTERVIEW DETAILS				
I1	Parish of Site or Event: CIRCLE ONE	<u>SOUTHEAST</u> KINGSTON 1 ST ANDREW 2 ST CATHERINE 3 ST THOMAS 4	<u>NORTHEAST</u> ST. MARY 5 ST ANN 6 PORTLAND 7	<u>WESTERN</u> WESTMORELAND 8 HANOVER 9 ST JAMES 10 TRELAWNY 11	<u>SOUTHERN</u> ST ELIZABETH 12 CLARENDON 13 MANCHESTER 14
I2	Record the name of the community in which the site or event is located		Name of Community:		
I3	Original Site Name		Name of Site:		
I4	NAME OF REPLACEMENT SITE, IF NECESSARY		Name of Site:		
I5	TYPE OF SITE	BAR 1 SPORTS BAR 2 NIGHT CLUB 3 EXOTIC CLUB WITH DANCERS 4 MASSAGE PARLOR 5 FAST FOOD / RESTAURANT 6 STREET SW SITE 7 HOTEL/ MOTEL /GUESTHOUSE 8 MALL/ PLAZA 9 OUTDOOR HANG OUT SPOT 10 OTHER (SPECIFY) 90 (go to I5b)		I5b. SPECIFY 90: _____	
I6	INTERVIEWER			INTERVIEWER INITIALS: _____	
I7	DATE TODAY			____/____/____ day month year	

I8	TIME INTERVIEW BEGINS	Hour: _____ Min: _____ Circle: AM or PM
I9	PERSON APPROACHED AS: CIRCLE ONE ANSWER.	STREET SEX WORKER 1 OTHER SEX WORKER (CLUB, MASSAGE PALOURS, ETC.) 2 DANCER 3 WORKER (BARTENDER, WAITRESS, ETC.) 4 PATRON 5
INTRODUCTION OF STUDY		
Hello, I am working on a study which is being conducted by the Jamaica Ministry of Health to identify where better health programs are needed. The study seeks to get information on sexual practices and sexually transmitted infections. I would like to offer you the opportunity to participate in the survey.		
ELIGIBILITY DETERMINATION		
E1	Sex of respondent (as identified by respondent) IF MALE, END INTERVIEW	MALE 1 FEMALE 2 TRANS WOMAN (Male to Female) 3
E2	ASK EVERYONE: What is your age? What is your date of birth? IF < 16, END INTERVIEW.	A. AGE IN YEARS: _____/ B. BIRTHDATE. /_____/_____/_____/_____ DAY MONTH YEAR
E3	IF AGE 16-17, Are you here with a parent or on a family errand?	END, NOT ELIGIBLE IF : YES 1 CONTINUE Q10A IF: NO 2

E4	ASK EVERYONE: Have you <u>ever</u> participated in a survey conducted by Ministry of Health?		CONTINUE Q10B IF: YES 1	
			REQUEST INFORMED CONSENT IF: NO 2	
E5	IF YES, Did you participate in a <u>Ministry of Health survey carried out around July 2017</u> in which persons got tested for HIV and <i>Syphilis</i> ?		END INTERVIEW IF: YES 1	
			REQUEST INFORMED CONSENT IF: NO 2	
No.	REQUEST INFORMED CONSENT. RESPONDENT MUST INITIAL CONSENT FORM			
E6	Just to confirm, I need to know if you are <u>willing</u> to....		YES	NO
	IF 'NO' TO ANY, END INTERVIEW.			
	A. ANSWER SURVEY QUESTIONS?		1	2
	B. GET AN HIV RAPID TEST?		1	2
	C. GET A SYPHILIS RAPID TEST?		1	2
	D. DO A STI SCREENING TEST		1	2
E7	IF NOT WILLING: Why not? END INTERVIEW IF NOT WILLING.		REASON: _____	
DEMOGRAPHIC INFORMATION				
D1	Parish of residence: CIRCLE ONE ANSWER	<u>SOUTHEAST</u> KINGSTON 1 ST ANDREW 2 ST CATHERINE 3 ST THOMAS 4	<u>NORTH EAST</u> ST. MARY 5 ST ANN 6 PORTLAND 7	<u>WESTERN</u> WESTMORELAND 8 HANOVER 9 ST JAMES 10 TRELAWNY 11
				<u>SOUTHERN</u> ST ELIZABETH 12 CLARENDON 13 MANCHESTER 14 OUTSIDE OF JAMAICA 15 Please note which country _____

D2	What is the highest grade you completed in primary school, in secondary school, and at post-secondary tertiary level? FILL IN NUMBER OF YEARS FOR EACH (A-D).	A. NONE B. PRIMARY C. All AGE D. SECONDARY E. HIGH SCHOOL F. TERTIARY (College or University) G. VOCACTIONAL/SKILLS TRAINING H. NO RESPONSE		
D3	Are you in school now?	YES 1 NO 2		
D4	IF WORKING, WHAT KIND OF WORK DO YOU DO?	DESCRIBE IN 1-2 WORDS. _____		
D5	Are you employed full-time, part-time, or not at all?	FULL-TIME 1 PART-TIME 2 NOT EMPLOYED 3		
No.	REASON FOR BEING AT SITE			
R1	Why did you come here today/tonight? Did you come to ... CIRCLE YES OR NO FOR EACH.		YES	NO
		A. TO SOCIALIZE	1	2
		B. TO DRINK ALCOHOL?	1	2
		C. TO WORK?	1	2
		D. LOOK FOR A SEXUAL PARTNER?	1	2

R2	How often do you come to this site? READ OPTIONS CIRCLE ONE ANSWER.	EVERYDAY 1 4-6 TIMES PER WEEK 2 2-3 TIMES PER WEEK 3 1 TIME PER WEEK 4	2-3 TIMES PER MONTH 5 1 TIME PER MONTH 6 LESS THAN ONCE A MONTH 7 THIS IS MY FIRST VISIT 8
R3	How long have you been involved in Sex work?	LESS THAN A WEEK 1 LESS THAN A MONTH 2 2 TO 6 MONTHS 3	6 TO 12 MONTHS 4 1-5 YEARS 5 OVER 5 YEARS 6 OVER 10 YEARS 7
R4	What do you think are the main 3 reasons for you getting involved in sex work? Read aloud and circle Three Only	GOT PREGNANT AND DID NOT FINISH SCHOOL 1 COULDN'T FIND A JOB 2 POVERTY 3 CHILDREN TO CARE FOR 4 NO FINANCIAL SUPPORT FROM BABY FATHER 5 NO FAMILY SUPPORT 6	GREW UP WITHOUT PARENTS 7 GREW UP /SPENT TIME IN CHILDREN'S HOME 8 WAS SEXUALLY ABUSED AS A CHILD 9 HAVE A DRUG OR ALCOHOL PROBLEM 10 WANTED TO BUY A CELL PHONE OR OTHER EXPENSIVE ITEM 11 RAN AWAY FROM HOME 12
R3	How many other public places have you been to today to socialize, drink alcohol or look for a person to have sex with? How many other public places do you plan to go to today or tonight to socialize, drink alcohol or look for a person to have sex with? PUT "0" IF NO OTHER PLACES.	A. OTHER PLACES BEEN TO: ____ B. OTHER PLACES WILL GO TO: ____	

THE NEXT FEW QUESTIONS ARE ABOUT SEXUAL PRACTICES		
S1	Have you ever had sex? IF NO, SKIP TO F8.	YES 1 NO 2
S2	How old were you the first time you had sex?	AGE FIRST HAD SEX : _____
S3	Do you have a main <u>male</u> sexual partner now? Do you have a main <u>female</u> partner now?	YES 1 NO 2 A. MAIN MALE PARTNER 1 2 B. MAIN FEMALE PARTNER 1 2
S4	Are you currently married or living with a partner? IF NO: Have you ever been married or lived with a sexual partner?	CURRENTLY MARRIED / LIVING WITH 1 PREVIOUSLY MARRIED/LIVED WITH, NOT NOW 2 NEVER MARRIED OR LIVED WITH 3
S5	What is the age of your oldest child? Specify month and year of birth of your oldest child.	AGE: _____ MONTH: _____ YEAR: _____ IF NO CHILD CIRCLE: 99
S6	The last time you had sex with your (most recent) main partner; did you and your partner use a condom? (Main partner is a live in partner or a visiting relationship)	YES NO YES 1 NO 2 NO MAIN PARTNER 9

S7	What were the main reasons for you not using condoms all the time with your main partner?	Love or trust partner 1 Don't like using condoms 2 Did not have one/ Could not get one 3 Partner does not like using condoms 4 Did not feel to use one/could not bother 5 Allergic to condoms/allergic to some brands 6 Trying to have a child 7 Using the pill /other contraceptive method 8 Have only one partner 9 Other Reason(s) 11 (go to S5b) S5b: Specify other _____								
S8	Do you believe that your main partner (any of your main partners) has had sex with another person in the past 6 months?	<table> <tr> <td>YES</td> <td>NO</td> </tr> <tr> <td>YES</td> <td>1</td> </tr> <tr> <td>NO</td> <td>2</td> </tr> <tr> <td colspan="2">NO MAIN PARTNER 9</td> </tr> </table>	YES	NO	YES	1	NO	2	NO MAIN PARTNER 9	
YES	NO									
YES	1									
NO	2									
NO MAIN PARTNER 9										
S9	In the past <u>12 months</u> , have you had sex with a person who you never had sex with before?	<table> <tr> <td>YES</td> <td>1</td> </tr> <tr> <td>NO</td> <td>2</td> </tr> </table>	YES	1	NO	2				
YES	1									
NO	2									
S10	Now I am going to ask you a few more questions about the number of persons you have had sex with. In total, how many persons did you have sex with in the past 7 days? This includes all the persons you have had sex with even if just one time.	TOTAL IN PAST 7 DAYS: _____								
S11	In total, how many persons did you have sex with in the past 4 weeks? This includes all the persons you have had sex with since (State the date one month before day of interview) . THIS NUMBER SHOULD BE THE SAME OR GREATER AS S10. IF NOT, PROBE!	TOTAL PAST 4 WEEKS: _____								

S10	Of the _____ (INSERT FROM S9) persons you had sex with in the past 4 weeks, how many of these were new persons you had never had sex with before? That is, you had sex with them for the first time in the past 4 weeks. THIS NUMBER SHOULD BE THE SAME OR <u>LESS</u> AS IN S9. IF NOT, PROBE!	NEW IN PAST 4 WEEKS: _____
S11	In total, with how many persons did you have sex with in the past 12 months? This includes all the persons you have had sex with. HELP PERSON ESTIMATE.	TOTAL IN PAST 12 MONTHS: _____
S12	Do you have a (male) condom with you now? IF YES: May I see it?	YES, CONDOM SEEN 1 YES, CONDOM NOT SEEN 2 NO 3
THE NEXT SET OF QUESTIONS ARE ON SEXUAL AND REPRODUCTIVE HEALTH AND FAMILY PLANNING		
F1	How many times have you been pregnant? Put "0" if never pregnant and skip to F8?	_____
F2	How many children do you have who were born alive?	_____
F3	A Pap Smear is a test for cancer of the cervix which is done during a pelvic examination by a doctor or nurse. How long has it been since your last Pap smear, if ever? IF NEVER HAD PAP SMEAR OR NOT SURE, SKIP TO F10	Never had one 1 Less than 1 year 2 More than 1 year, less than 2 years ago 3 2 to 3 years 4 More than 3 years ago 5 Does not know / not sure 6

F4	In your last 10 sexual experiences, how many times was a condom used? READ OPTIONS If condom used all 10 times, go to F 13	All 10 times 1 8-9 times 2 7-6 times 3 5-3 times 4 1-2 times 5 Never use a condom at all 5 Not applicable 9								
F5	Are you and your partner <u>currently</u> (last 30 days) using a method of contraception or doing anything to prevent pregnancy? If 'No' got to C1	YES 1 NO 2								
F6	If yes, which method(s) are you currently using? Tick all that apply	Female sterilization, tubal ligation (a.k.a <i>tie-off</i>) 1 Implant (Jadelle) 3 IUD/ Coil 4 Injection (Depo) 5 Pill 6 Condom 7 Emergency Contraceptive Protection (a.k.a. the Morning After Pill) 8 Spermicides 9 Withdrawal 10 Rhythm, Calendar, Billings (a.k.a the safe period) 11 Other (go to F13b) 12 F13b. Specify other: _____								
COMMERCIAL SEX AND CONDOM USE										
C1	ASK EVERYONE WHO HAS EVER HAD	<table border="1"> <tr> <td data-bbox="667 1745 708 1808"></td> <td data-bbox="708 1745 1312 1808"></td> <td data-bbox="1312 1745 1419 1808">YES</td> <td data-bbox="1419 1745 1552 1808">NO</td> </tr> <tr> <td data-bbox="667 1808 708 1871">A</td> <td data-bbox="708 1808 1312 1871">Ever received help with expenses?</td> <td data-bbox="1312 1808 1419 1871">1</td> <td data-bbox="1419 1808 1552 1871">2</td> </tr> </table>			YES	NO	A	Ever received help with expenses?	1	2
		YES	NO							
A	Ever received help with expenses?	1	2							

	SEX. Sometimes people have sex with someone in exchange for gifts or help with expenses or cash money. Have you....		B	Ever <u>received</u> money in exchange for sex?	1	2
			C	Ever paid money for sex?	1	2
			D	Ever received gifts or favors in exchange for sex?	1	2
C2	If <u>RECEIVED</u> CASH MONEY: Have you RECEIVED cash money by a person for sex <u>in the past 3 months</u> ? If No to both C2 A& B, skip to C9			YES NO NA A. PAST 3 MONTHS? 1 2 9 B. PAST 6 MONTHS? 1 2 9		
C3	In which parishes did you receive cash for sex in the <u>past 6 months</u> ? CIRCLE 1	<u>SOUTHEAST</u> KINGSTON 1 ST ANDREW 2 ST CATHERINE 3 ST THOMAS 4	<u>NORTHEAST</u> ST. MARY 5 ST ANN 6 PORTLAND 7	<u>WESTERN</u> WESTMORELAND 8 HANOVER 9 ST JAMES 10 TRELAWNY 11	<u>SOUTHERN</u> ST ELIZABETH 12 CLARENDON 13 MANCHESTER 14 OUTSIDE OF JAMAICA 15 Which countries? 	
C4	In the past <u>3 months</u> , at how many different places did you look for men to pay you money for sex?			NUMBER OF SITES: _____		
C5	Is Sex work your main source of income?			YES 1 NO 2		
C6	The last time you had sex with a new paying client, did you and your partner use a condom?			YES 1 NO 2		

C7	The last time you had sex with a regular paying client, did you and your partner use a condom?	YES 1 NO 2 NOT APPLICABLE 9
C8	Has a condom ever slipped off during sex in the last 4 weeks?	YES-ONCE OR TWICE 1 YES- 3-5 TIMES 2 YES-MORE THAN 5 TIMES 3 NO-NEVER 4 DOES NOT REMEMBER 9
C9	Has a condom ever burst while you were having sex during the last 4 weeks?	YES-ONCE OR TWICE 1 YES- 3-5 TIMES 2 YES-MORE THAN 5 TIMES 3 NO-NEVER 4 DOES NOT REMEMBER 9
C10	Have you ever used a lubricant during sex in the last 4 weeks?	EVERY TIME 1 ALMOST EVERY TIME 2 SOMETIMES 3 NEVER 4 (go to Sy 1) DOES NOT REMEMBER 9
C11a	If Yes, do you use lubricant with condom for vaginal sex, anal (Sex in the bottom) sex or both?	Vaginal 1 Anal 2 Both 3 Other 4 (go to C11b)

		C11b: Specify Other: _____			
C12 a	Have you ever used any of the following lubricant products during sex in the last 3 months? Tick all that apply	Vaseline or other petroleum jelly product 1 KY Jelly or other water-based product 2 EVE 3 Life Style 4 Saliva or water 5 Baby or body lotion 6	Soap 7 Vegetable or food oils 8 Butter or margarine 9 I don't use lubricant 10 Other 11 (go to C13b) Don't remember 99 Refuse to Answer 97 C13b: Specify Other: _____		
CURRENT SYMPTOMS AND PARTICIPATION IN HEALTH PORGRAMS AND TESTING: ASK EVERYONE					
Sy1	A. Have you had a genital discharge, ulcer, sore, warts, pain or burning when you urinate anytime <u>in the past 3 months</u> ? B. IF NO: Anytime <u>in the past 12 months</u> ? IF NO: CIRCLE 3's IN Sy2.		YES NO A. PAST 3 MONTHS? 1 2 B. PAST 12 MONTHS? 1 2		
Sy2	Which of the following did you do the last time you had a discharge, ulcer or sore? READ EACH.		YES	NO	N/A
	A. GET DRUGS WITHOUT PRESCRIPTION?		1	2	3
	B. GO TO A PUBLIC HEALTH CARE PROVIDER?				
	C. GO TO A PRIVATE DOCTOR?		1	2	3
	D. TREAT WITH A HOME REMEDY		1	2	3

		E. HAVE SEX WHILE HAD SYMPTOMS?	1	2	3
Sy3	Have you been told by a health care worker in the past 12 months (SINCE MONTH YEAR) that you had a sexually transmitted infection? IF YES, CIRCLE one answer. B.If NO, Have you <u>ever</u> been told? CIRCLE 2 IF EVER or 3 IF NEVER.		YES, PAST 12 MONTHS 1	YES, BUT NOT IN PAST 12 MONTHS 2	NEVER 3
Sy4	a. Do you know where you can go if you wish to receive an HIV test? b. Have you ever been tested for HIV? If Never tested skip to SY8		YES 1	NO 2	Never had a test 3
Sy5	Not including today, have you had an HIV test in the past 12 months? CIRCLE on		YES, IN PAST 12 MONTHS 1	YES, EVER HAD A TEST 2	
Sy6	The last time you were tested, did you get your result? If Yes, were you counselled when you got your result?		DID NOT GET RESULT 1	GOT RESULT AND WAS <u>COUNSELED</u> 2	GOT RESULT BUT <u>NOT COUNSELED</u> 3
				NEVER TESTED 9	

Sy7	ASK BOTH OF THESE QUESTIONS: Did you tell your main partner the results of your last HIV test (not including the test today)? CIRCLE one answer. Did you tell any of your friends, family or co-workers the results of your last HIV test (not including the test today)?	A. MAIN PARTNER? YES 1 NO 2 NA 9 B. ANY FRIENDS, FAMILY? YES 1 NO 2 NA 9
Sy8a	What is the main reason why you have never gotten tested for HIV?	Don't Think I Am At Risk 1 Never thought about it 2 Fear of Positive Result 3 No Money or Time to Get Tested 4 Fear that my results will not be kept confidential by health care workers 5 Fear of Health Care Workers treating me badly 6 Didn't know I could get tested for HIV 7 Other reasons 9 (go to Sy8b) Sy8b: Specify other: _____

A FEW QUESTIONS ON SUBSTANCE ABUSE				
A1	During the last 4 weeks how often have you had drinks containing alcohol? Would you say daily, weekly, less than weekly, or never?. CIRCLE ONE CODE.			DAILY 1 WEEKLY 2 LESS THAN WEEKLY 3 NEVER 4
A2	In the past 6 months since (MONTH YEAR), have you..... ASK EACH. CIRCLE YES OR NO FOR EACH.		YES	NO
		A. INJECTED ANY NON-PRESCRIPTION DRUGS?	1	2
		B. USED CRACK OR COCAINE?	1	2
		C. USED GANJA?	1	2
		D. USED ECSTACY?	1	2
A FEW QUESTIONS ABOUT VULNERABILITY				
A3	Have you <u>ever</u> :		YES	NO
		A.SPENT A NIGHT IN A JAIL OR PRISON?	1	2
		B.BEEN RAPED?	1	2
		C.SLEPT OUTDOORS OR IN A SHELTER BECAUSE YOU WERE HOMELESS?	1	2
		D. BEEN VIOLENTLY ATTACKED BY A CLIENT	1	2
		E. BEEN VIOLENTLY ATTACKED BY A SEX PARTNER	1	2
		F. BEEN VIOLENTLY ATTACKED BY ANOTHER SEX WORKER	1	2
EXPOSURE TO HIV/STI PREVENTION INTERVENTIONS				
A4	I want to know if you have been a part of any		YES	NO
		A. PARTICIPATED IN A CONDOM DEMONSTRATION?	1	2

	HIV/AIDS prevention programs in the past 6 months. In the past 6 months have you.....	B. SEEN ANY AIDS PREVENTION MATERIALS (e.g. Posters, brochures, cards) AT YOUR CURRENT WORKSITE?		1	2
		HAVE YOU TALKED WITH AN OUTREACH WORKER <u>AT YOUR CURRENT WORK SITE ABOUT...?</u>			
		C. YOUR RISK OF GETTING HIV/AIDS	YES	NO	NA
		1	2	9	
	D. GETTING TESTED FOR HIV?	1	2	9	
	Do you think...?	YES	NO	DK	
A5	A. Using condoms reduce the risk of HIV transmission?	1	2	9	
	B. A healthy looking person can be infected with HIV?	1	2	9	
	C. A person can get HIV from mosquito bites?	1	2	9	
	D. A person can get HIV by sharing a meal with someone who is infected?	1	2	9	
	E. Having sex with only one faithful, uninfected partner can reduce the risk of HIV transmission?	1	2	9	
NOW I WOULD LIKE TO ASK YOU A FEW MORE QUESTIONS TO FINISH THIS PART OF THE SURVEY.					
A6	How much risk do you think you have of getting HIV?	Great Risk 1 Moderate Risk 2 Little Risk 3 No Risk at All 4 Does not Know 5			

READ: I would like you to answer a few questions privately on a card that I will give you. I will read the question and you should write down your honest answer using a pen that I will give you. I'll read the questions and the answers. You should choose an answer and circle your response on the card. When we are finished with the card, you fold it and put it in this envelope. I cannot see your answers. Nobody can link the responses to you so there is no reason to give a false answer. We really want to know the truth. Some of the questions may seem repetitive but please be patient.

P1	ASK EVERYONE: Are you willing and able to mark your answers on the card?	YES 1 NO 2
P2	The last time you had sex with your main partner, did you and your partner use a condom? IF YOU DO NOT HAVE A MAIN PARTNER, CIRCLE NOT APPLICABLE.	YES 1 NO 2 NOT APPLICABLE 9
P3	In the past 3 months, have you paid someone money for sex?	YES 1 NO 2 NOT APPLICABLE 9
P4	In the past 3 months, have you <u>been paid</u> money for sex?	YES 1 NO 2 NOT APPLICABLE 9
P5	The last time you paid or were paid money for sex, did you use a condom?	YES 1 NO 2 NOT APPLICABLE 9
P6	In the past 6 months have you had sex with only men, only women, or both men and women?	1. ONLY MEN 2. ONLY WOMEN 3. BOTH MEN & WOMEN 4. HAVE NOT HAD SEX IN PAST 6 MONTHS

P7	What types of sex have you had in the past year? Have you had oral sex? Anal sex (Sex in the bottom)? Penis to vagina?	1. ORAL 2. ANAL 3. PENIS TO VAGINA 5. NONE
P8	The last time you had penis to vaginal sex, did you or your partner use a condom?	1. YES 2. NO 3. NEVER PENIS TO VAGINA SEX 9. NOT APPLICABLE
P9	The last time you had anal sex (Sex in the bottom); did you or your partner use a condom?	1. YES 2. NO 3. NEVER ANAL SEX 9. NOT APPLICABLE
P10	In the past 12 months, have you had sex with more than one person?	1. YES 2. NO 3. NOT APPLICABLE
P11	What was the result of your most recent HIV test not including the test today?	1. POSITIVE 2. NEGATIVE 3. NEVER TESTED 4. DON'T KNOW/DID NOT GET RESULTS
P12	If your result was positive , have you ever registered for care and treatment at any health provider since your positive result? IF RESULT WAS NEGATIVE OR NEVER TESTED CIRCLE 9 FOR NOT APPLICABLE	YES-PRIVATE DOCTOR 1 YES-PUBLIC HEALTH CENTRE 2 YES-BOTH PUBLIC AND PRIVATE DOCTORS 3 NO 4 NOT APPLICABLE 9
P13	If you received a HIV positive result and have registered for care and treatment of HIV , have you visited a clinic/doctor for treatment in the last 12	YES 1 NO 2

	months	NOT APPLICABLE 9
P14	If No, what are the reasons for you not going to a doctor or clinic in the last 12 months?	FEEL HEALTHY 1 FEAR BEING TREATED BADLY BY CLINIC STAFF/HEALTH CARE WORKERS 2 DON'T WANT OTHERS TO KNOW OF MY INFECTION 3 TOO LONG WAITING TIME AT CLINIC 4 NOT APPLICABLE 9
P15	How long after receiving your positive HIV result did you go to a clinic/doctor?	WITHIN ONE MONTH 1 WITHIN 3 MONTHS 2 WITHIN ONE YEAR 3 LONGER THAN ONE YEAR 4 NOT APPLICABLE 9
P16	What is the main reason for you not registering at a clinic or doctor after receiving your positive HIV result?	FEEL HEALTHY 1 FEAR BEING TREATED BADLY BY CLINIC STAFF/HEALTH CARE WORKERS 2 DON'T WANT OTHERS TO KNOW OF MY INFECTION 3 TOO LONG WAITING TIME AT CLINIC 4 NOT APPLICABLE 9
Thank you very much for participating. Now you will be tested for HIV and Syphilis		
TIME ENDED: HOUR: __/__ MINUTE __/__ AM / PM Checked by Supervisor: _____ Date: _____ Entered by Data entry person: _____		

INTERVIEWER COMMENTS.		
ORIGINAL Q8	PERSON CONFIRMED AS:	1. STREET SEX WORKER 2. OTHER SEX WORKER (CLUB, MASSAGE PALOURS, ETC.) 3. WORKER (BARTENDER, WAITRESS, ETC.) 4. PATRON

FORM D: WORKER AND PATRON CONFIDENTIAL SLIP

STICKER: _____

CARD 1: RESPONDENT VERSION

A1	YES	NO	NOT APPLICABLE
A2	YES	NO	
A3	YES	NO	
A4	YES	NO	NOT APPLICABLE
A5	ONLY MEN	ONLY WOMEN	BOTH MEN & WOMEN
A6	ORAL	ANAL	PENIS TO VAGINA
A7	YES	NO	NEVER PENIS TO VAGINA
A8	YES	NO	NEVER ANAL SEX
A9	POSITIVE	NEGATIVE	NEVER TESTED
A10	YES	NO	
Thank-you for answering these questions. Please put this card in the envelope			

FORM E: SUPERVISOR REPORT SHEET

Fill one per recruitment effort (A recruitment effort is each time participants are sought, such as at a site such as a bar is visited. If a venue is visited more than one time, fill out a separate Supervisor Report Form for each visit.)

The 4th (2017) Generation Surveillance of Commercial Sex Workers Female Patrons and Workers of Sites where Persons Meet Sex Partners or Participate in Sexual Activity in Jamaica

Questions		Response Categories			
PLACE_ID ____ / ____		/ / Day Month Year			
Name of site:					
TYPE: CIRCLE ONE:		STREET	DANCE_CLUB	EVENT	OTHER
	TARGET NUMBER	NUMBER AT ARRIVAL	NUMBER AT DEPARTURE	NUMBER APPROACHED	NUMBER COMPLETED
STREET SEX WORKERS:					
OTHER FEMALE SEX WORKERS:					
FEMALE WORKERS (BARTENDER , ECT.):					
FEMALE PATRONS:					
TOTAL:					
HOW SUPPORTIVE WAS THE GATEKEEPER? ENTHUSIASTIC WILLING REFUSED NA					
HOW WILLING WERE WORKERS TO PARTICIPATE? ENTHUSIASTIC WILLING REFUSED NA					
HOW WILLING WERE PATRONS TO PARTICIPATE? ENTHUSIASTIC WILLING REFUSED NA					
Visit to place for recruitment: 1st 2nd 3rd			Number of interviewers working:		
QUESTIONNAIRE NUMBERS USED: LIST HERE					

FORM F: BATCH SHEET

NAME OF COORDINATOR: _____

The 4th (2017) Generation Surveillance of Commercial Sex Workers Female Patrons and Workers of Sites where Persons Meet Sex Partners or Participate in Sexual Activity in Jamaica

PLACE_ID : _____

PLACE_ID: _____

PLACE_ID : _____

PLACE_ID: _____

NUMBER OF QUESTIONNAIRES: _____

DATE GIVEN TO DATA ENTRY: _____

QUESTIONNAIRE ID CODES: _____

FORM G: MOH CONFIDENTIALITY AGREEMENT

OFFICIAL SECRETS ACTS

DECLARATION

Declaration to be signed by Public Officers on Appointment

My attention has been drawn to the provisions of the Official Secrets Acts, 1911 and 1920, which are set out on the back of this document, and I am fully aware of the serious consequences which may follow any breach of those provisions.

I understand that the sections of the Official Secrets Acts set out on the back of this document, cover articles published in the Press or in book form, and I am aware that I must not divulge any information gained by me as a result of my employment to any unauthorized person, orally or in writing, without the previous sanction of the head of the Department. I understand also that these provisions apply not only during the period of my employment but also after my employment in the public service has ceased.

Signed

Witness

Date

Extracts from the Official Secrets Acts, 1911 and 1920

Section 2 of the Official Secrets Act, 1911, as amended by the Official Secrets Act, 1920, provides as follows:—

“2(1)—If any person having in his possession or control any secret official code word or pass word or any sketch, plan, model, article, note, document, or information which relates to or is used in a prohibited place or anything in such a place, or which has been made or obtained in contravention of this Act or which has been entrusted in confidence to him by any person holding office under Her Majesty or which he has obtained or to which he has had access owing to his position as a person who holds or has held office under Her Majesty, or as a person who holds or has held a contract made on behalf of Her Majesty, or as a person who is or has been employed under a person who holds or has held such an office or contract:—

- (a) communicates the code word, pass word, sketch, plan, model, article, note, document, or information to any person other than a person to whom he is authorized to communicate it, or a person to whom it is in the interests of the State his duty to communicate it; or
 - (aa) uses the information in his possession for the benefit of any foreign power or in any other manner prejudicial to the safety or interests of the State; or
 - (b) retains the sketch, plan, model, article, note or document in his possession or control when he has no right to retain it or when it is contrary to his duty to retain it or fails to comply with all directions issued by lawful authority with regard to the return or disposal thereof; or
 - (c) fails to take reasonable care of, or so conducts himself to endanger the safety of the sketch, plan, model, article, note, document, secret official code or pass word, or information;
- that person shall be guilty of a misdemeanour.

(1A) If any person having in his possession or control any sketch, plan, model, article, note, document, or information which relates to munitions of war, communicates it directly or indirectly to any foreign power or in any other manner prejudicial to the safety or interests of the State, that person shall be guilty of a misdemeanour.

(2) If any person receives any secret official code word or pass word or sketch, plan, model, article, note, document, or information, knowing or having reasonable ground to believe, at the time when he receives it, that the code word, pass word, sketch, plan, model, article, note, document, or information is communicated to him in contravention of this Act, he shall be guilty of a misdemeanour, unless he proves that the communication to him of the code word, pass word, sketch, plan, model, article, note, document, or information was contrary to his desire.”

Section 1 (2) of the Official Secrets Act, 1920, provides as follows:—

“(2) If any person—

- (a) retains for any purpose prejudicial to the safety or interests of the State any official document, whether or not completed or issued for use, when he has no right to retain it, or when it is contrary to his duty to retain it, or fails to comply with any directions issued by any Government Department or any person authorized by such department with regard to the return or disposal thereof; or
 - (b) allows any other person to have possession of any official document issued for his use alone, or communicates any secret official code word or pass word so issued, or, without lawful authority or excuse, has in his possession any official document or secret official code word or pass word issued for the use of some person other than himself, or on obtaining possession of any official document by finding or otherwise, neglects or fails to restore it to the person or authority by whom or for whose use it was issued, or to a police constable; or
 - (c) without lawful authority or excuse, manufactures or sells, or has in his possession for sale any such die, seal or stamp as aforesaid,
- he shall be guilty of a misdemeanour.”

S.P.O.

FORM H: INFORMED CONSENT FORM FOR PARTICIPANTS (HIV AND *SYPHILIS* TESTING)

Introduction

Thank you for agreeing to speak with me today. My name is _____. I am part of an assessment team organized by the Ministry of Health to conduct a survey to get needed information to provide more effective HIV/STIs prevention and control programs for sex workers and other persons at risk of contracting sexual transmitted infections.

Should you have any questions or concerns about this initiative and/or the people involved, or if you experience any discomfort as a result of the study, please contact: Dr. Nicola Syers or Ms. Marion Scott, who are the Principal Investigators at the Ministry of Health at 633-8172. For independent advice on your rights as a research participant please contact Prof. Owen Morgan, Chairman, Advisory Panel on Ethics & Medico-Legal Affairs at (876) 967-1100/3/5/7.

Purpose

The Ministry of Health is implementing the survey to learn more about HIV-related risk behavior among sex workers and patrons and also to measure the effectiveness of prevention currently being offered. What we learn from the survey will help us better plan and coordinate prevention, treatment, and care services for sex workers, in partnership with other stakeholders.

We want your input, advice and help. I will ask you some questions about sexual behavior and request of you to get tested for HIV and *Syphilis*.

Optional Participation and Protection of Privacy

- Your participation is voluntary.
- I will not collect your name or anything else that describes your identity.
- The answers that you give to questions will never be linked to your name.
- Your opinions will remain confidential.
- There may be questions that I ask that you deem of a personal nature. Please know that you can refuse to answer any question and you can time
- The input that you provide today on the questionnaire will be kept in a locked filing cabinet only accessible to limited trained survey staff.
- You are not required to answer any question unless you want to.
- You can stop your participation in the discussion any time you want to.

Benefits for Participation

Benefits to the participants include getting tested for HIV and *Syphilis*, and referral for treatment, if required. Information on test results from previous surveillance surveys has been collected. It is possible that some of participants in this 2014 survey were tested in previous surveys. Hence, we will ask about your history of participating in previous surveillance survey.

If you have been tested in previous surveillance surveys, we will ask for your permission to look up your results in order to compare them with the new test results.

Participants may also benefit from the programs if they are still in the community when the programs are implemented. Benefits to society are potentially great as results will be used by stakeholders such as the National HIV/STI Program of the Ministry of Health and the NFPB-SHA to design HIV prevention interventions for sex workers, high risk women and for the general population. The the NFPB-SHA will use the data to improve improve programs in the area of HIV and STI prevention and control.

Possible Risks of Participation

There is minimal risk of breach of confidentiality from this study. Confidentiality of subjects will be maintained and protected by the use of numbers to label study specimens, case report forms and other study forms. Study documents including consent forms with the subjects' names, will be stored in a separate logbook in a locked cabinet at the NFPB-SHA. *Syphilis* and HIV are reportable diseases in Jamaica. All HIV and *Syphilis* results will be entered into the MOH surveillance system using the standard system to maintain confidentiality of patient results. To ensure confidentiality and minimize risks to participants, all HIV and *Syphilis* testing will be conducted by staff already employed to the National HIV/STI Programme or the Regional Health Authorities who conduct outreach VCT for HIV and *Syphilis* as part of their routine function.

Time Commitment

This interview should last for 30-40 minutes.

Are you willing to participate? If so, please read:

I have been fully informed about the purpose of the interview. I fully understood the contents of this form.

I consent to participate in the interview.

Do you have any questions before we start?

Mark (X) on the appropriate response.

I Agree _____

I Disagree _____

Witness

Name of interviewer _____

Signature _____ Date _____

